I want to build a portal for Occu-Med's employees and Applicant's goimg through the RDQA Process to have additional recources that they need to address their RDQA condition. Give me SMART AS FUCK IDEAs that we can include so that wer can then pitch it to Andy. It should help cut cost (need for addiutional Exam QA analysts, less time spent by EQA Anayst babysittig them and explaining everything from scratch) and give the company an avenue to charge the clients mpre

This is a strong pitch idea, Alex — and the key is framing it as **not just a "resource portal," but a cost-control + client-value product**. I'll structure this like something you can lift directly into an Andy pitch.
Thought for a couple of seconds
Hey Alex — yes. This could be **really valuable** if you position it correctly.
The smart pitch is:
Occu-Med can reduce RDQA hand-holding, standardize applicant guidance, shorten turnaround time, and create a premium client-facing service by building an RDQA Support Portal.
Not "just a portal."
A **guided RDQA resolution system.**

RDQA Support Portal: Smart Features to Pitch to Andy

1. Condition-Specific RDQA Guidance Pages

Instead of EQA analysts explaining the same thing over and over, the portal would have simple condition pages for the most common RDQA triggers.
Examples:

Condition	Portal Page Includes
Hypertension	What BP readings are needed, acceptable documentation, provider letter guidance
Diabetes	A1c expectations, medication list, complications screening, provider clearance language
Asthma	Control status, rescue inhaler use, pulmonary symptoms, restrictions/accommodations
Sleep Apnea	CPAP compliance report, treatment status, provider statement

Mental Health History	Stability, medications, hospitalizations, restrictions, provider documentation
Orthopedic Conditions	Range of motion, restrictions, imaging, functional limitations
Cardiac History	Stress test, cardiology follow-up, symptoms, medication stability

Each page should answer:
What triggered the RDQA?
What does Occu-Med need?
What should the provider address?
What documents should be uploaded?
What common mistakes delay review?
This alone could cut a massive amount of repetitive explanation.

2. "What Do I Need to Send?" Interactive Checklist

Applicant selects their condition:
"I was delayed for diabetes."
The portal generates a customized checklist:

Diabetes RDQA Checklist

Required items may include:

- Most recent A1c result
- Current medication list
- Provider statement confirming stability
- Documentation of complications, if any
- Whether the individual has neuropathy, kidney disease, vision issues, or cardiovascular complications
- Whether any restrictions or accommodations are needed
- Whether the condition impacts ability to safely perform essential job duties

Then the portal shows a status tracker:

Item	Status
A1c result	Missing
Provider statement	Uploaded
Medication list	Missing
Restrictions addressed	Incomplete

This prevents applicants from sending random, incomplete documents.

3. Provider Letter Generator

This is one of the smartest ideas.
Instead of applicants going to their doctor with vague instructions, the portal

generates a **condition-specific provider guidance letter**.
Example:
"Please address the following items regarding the applicant's hypertension: current diagnosis, recent blood pressure readings, medication regimen, stability, symptoms, complications, restrictions, and whether the condition affects the individual's ability to safely perform the essential duties of the position."
This would help because providers often write useless notes like:
"Patient is cleared."
That does not help Occu-Med.
The portal should explain:
Occu-Med is not asking the provider to clear the applicant. We are asking the provider to document current medical status so Occu-Med's review team can complete the evaluation.
That wording matters.

4. Upload Portal With Document Quality Checks

Applicants should be able to upload documents directly.
But not just upload. The portal should guide them.
Before submission, the system asks:

- Does the document include your name?
- Does it include the provider's name?
- Is it dated?
- Is it legible?
- Does it address the requested condition?
- Does it mention restrictions or accommodations?
- Does it include test results, if requested?

Then it warns them:
"This document may be incomplete because it does not appear to include a provider signature/date."
Even basic checks could reduce a ton of back-and-forth.

5. "Why Was I Delayed?" Simple Explanation Tool

Applicants get overwhelmed when told they are RDQA.
The portal should have a plain-English explanation:
"RDQA does not mean you are disqualified. It means Occu-Med needs more information before a final recommendation can be made."
Then it explains:

- Why additional information is needed
- Who reviews the information
- What the applicant needs to do
- What Occu-Med can and cannot tell them
- Why incomplete records cause delays

This reduces panic calls and angry emails.

6. Applicant Timeline Tracker

The portal should show where the applicant is in the RDQA process:

Example RDQA Timeline

Step	Status
RDQA issued	Complete
Applicant notified	Complete
Records requested	Complete
Applicant uploaded documents	Pending
EQA review	Not started
SME review	Not started
Final recommendation	Not issued

This helps stop the constant:
"Any update?"
"Did you get my records?"
"What happens next?"
It gives applicants visibility without requiring EQA to manually explain everything.

7. Automated "Missing Information" Messages

When EQA marks something incomplete, the portal can generate a clean message.
Example:
"Thank you for submitting your records. We are still missing documentation addressing whether your condition requires restrictions, accommodations, or work modifications. Please have your provider clarify this information."
This would reduce time spent typing the same thing repeatedly.
Even if the portal does not send automatically at first, it could generate text for EQA to copy/paste.

8. Internal EQA Dashboard

This is where Andy will care.
The portal should have an internal side for employees.
EQA analysts could see:

Applicant	Condition	Missing Items	Last Contact	Days Pending	Risk Level
John Smith	Diabetes	A1c, provider statement	2 days ago	5	Medium
Jane Doe	Sleep Apnea	CPAP report	1 day ago	3	Low

Useful filters:

- Pending applicant action
- Pending EQA review
- Pending SME review
- Missing documents
- High delay risk
- Client priority
- Condition type

This makes RDQA case management less chaotic.

9. Client Dashboard / Premium Client Add-On

This is how Occu-Med can charge more.
Clients could get a dashboard showing:

- Number of applicants in RDQA
- Average RDQA turnaround time
- Common delay reasons
- Number of cases pending applicant action
- Number of cases pending provider documentation
- Which conditions are causing the most delay
- Monthly RDQA analytics

Example:

Client RDQA Insights

Metric	Value
Total RDQA cases this month	42
Average resolution time	6.8 days
Most common delay	Missing provider statement
Most common condition	Hypertension
Cases pending applicant action	18
Cases pending medical review	7

This becomes a **premium reporting product**, not just an internal tool.
Pitch phrase:
"This would allow Occu-Med to offer clients a higher-touch RDQA management package with visibility, analytics, and faster case movement."

10. Client-Branded RDQA Resource Pages

For major clients, Occu-Med could offer customized portals.
Example:
V2X RDQA Resource Center
KBR Medical Follow-Up Portal
Amentum RDQA Guidance Hub
Each one could include:

- Client-specific job categories

- Client-specific forms
- MOD 17-related guidance
- Deployment-specific medical standards
- Client-specific turnaround expectations
- Dedicated upload link
- Branded reporting dashboard

This makes the service feel more premium and justifies higher pricing.

11. RDQA “Smart Intake” Form

When an applicant gets RDQA, they complete a guided form before EQA spends time chasing them.

Example questions:

For Hypertension

- What was your most recent blood pressure reading?
- Are you currently taking medication?
- Has your medication changed recently?
- Do you have symptoms such as chest pain, dizziness, fainting, or shortness of breath?
- Has your provider placed any restrictions on you?
- Do you need any accommodations or work modifications?

The answers help EQA and SMEs decide what is actually needed.

This is huge because it collects structured information instead of messy emails.

12. “Provider Doesn’t Know What to Write” Help Section

This happens constantly.

The portal should have a section titled:

What Your Provider Should Include

And then condition-specific examples.

Example:

For sleep apnea:

- Diagnosis
- Treatment method
- CPAP use/compliance
- Current symptoms
- Daytime sleepiness
- Any restrictions
- Whether the condition is stable
- Whether additional follow-up is needed

Also include:

What Not to Submit

- A one-line note saying “cleared”
- Old records without current status

- Lab results without provider interpretation
- Screenshots that do not show the applicant's name/date
- Documents missing signature/date

This would reduce junk submissions.

13. RDQA Document Templates Library

Internal employees could access reusable templates:

- Applicant instruction templates
- Provider letter templates
- Client update templates
- Missing info templates
- Condition-specific follow-up templates
- Final document received confirmation
- Escalation templates

This helps newer EQA analysts and reduces training burden.
It also standardizes language across the company.

14. "RDQA Readiness Score"

This would be smart and impressive.
Each case gets a score:

Score	Meaning
0–40%	Not ready for SME review
41–70%	Partially complete
71–90%	Likely reviewable but may need clarification
91–100%	Ready for SME review

Example:

Diabetes RDQA Readiness Score: 62%

Missing:

- Updated A1c
- Provider statement addressing complications
- Restriction/accommodation statement

This helps prevent incomplete packets from being sent to SMEs too early.

15. Applicant "Next Best Action" Button

Instead of giving applicants a wall of text, the portal should always show one clear next action.
Example:
Your next step: Upload your most recent A1c lab result.
Or:
Your next step: Send the provider guidance letter to your treating physician.

This is ADHD-friendly, applicant-friendly, and reduces confusion.

16. AI Assistant Trained on Occu-Med RDQA Rules

This could be internal first, then applicant-facing later.
Internal EQA version:
“Applicant has diabetes, A1c 7.4, nephropathy noted. What follow-up items should we request?”
The assistant returns:

- Current A1c
- Provider statement on diabetes stability
- Kidney function status
- Complications
- Restrictions/accommodations
- Medication list
- Whether condition impacts essential job duties

Applicant-facing version should be more controlled:
“I was delayed for high blood pressure. What should I ask my doctor for?”
It gives simple guidance without giving medical advice.
Important disclaimer:
“This tool does not make medical determinations. It only helps you understand what documentation may be needed for Occu-Med’s review process.”

17. Escalation Detection

The portal could flag cases that are sitting too long.
Examples:

- Applicant has not uploaded documents after 5 days
- Provider note submitted twice but still incomplete
- Case is pending client approval
- Applicant submitted records that may indicate urgent concern
- Case needs SME review
- Case has approaching deployment deadline

This saves EQA from manually babysitting every case.

18. RDQA Analytics for Leadership

Andy would probably care about this part the most.
The system could show:

- Average RDQA resolution time by condition
- Which conditions cause the most delay
- Which clients generate the most RDQA cases
- Which providers submit incomplete documentation most often
- How much EQA time is spent on repeat explanations
- How many cases are delayed due to applicant inaction

- How many cases are delayed due to provider documentation issues
- Which templates are used most often

This turns RDQA from a messy workflow into measurable operations data.

19. Client Pricing Opportunities

This portal creates multiple ways to charge more.

Option 1: Premium RDQA Management Fee

Occu-Med charges clients an added fee for enhanced RDQA support.
Includes:

- Applicant portal access
- Guided condition-specific instructions
- Upload tracking
- Client dashboard
- Monthly reporting

Option 2: Per-RDQA Case Administrative Fee

Example:
"Enhanced RDQA coordination and documentation support: $X per case."

Option 3: Client Subscription Model

Large clients pay monthly for:

- Dashboard access
- Analytics
- Custom reporting
- Dedicated RDQA guidance library
- Priority case tracking

Option 4: White-Labeled Client Portal

Higher-tier clients get branded portals.
Example:
"V2X Medical Follow-Up Portal powered by Occu-Med."

Option 5: Faster Turnaround Package

Clients can pay for premium RDQA workflow support:

- Faster intake review
- More frequent applicant reminders
- Priority EQA queue
- Enhanced reporting

20. Best Pitch Framing for Andy

Use this framing:

Problem

RDQA cases require repeated manual explanation, applicant follow-up, provider clarification, document review, and status updates. This creates significant EQA workload and slows case resolution.

Solution

Build an RDQA Support Portal that gives applicants, employees, and clients guided access to condition-specific instructions, document checklists, upload tracking, provider guidance letters, and case-status visibility.

Business Value

The portal would:

- Reduce repetitive EQA communication
- Standardize applicant instructions
- Reduce incomplete provider documentation
- Shorten RDQA turnaround time
- Improve applicant experience
- Improve client transparency
- Create a premium service Occu-Med can charge for
- Reduce the need to add more EQA analysts as volume increases

The Strongest "Smart as Fuck" Ideas

If you want the most powerful pitch, focus on these:

1. **Condition-specific RDQA checklist generator**
2. **Provider letter generator**
3. **Applicant upload portal with missing-document detection**
4. **Internal EQA dashboard**
5. **Client-facing RDQA analytics dashboard**
6. **RDQA readiness score**
7. **Automated missing-information messages**
8. **Premium client add-on pricing**
9. **White-labeled client portals**
10. **AI assistant for EQA and applicants**

That combination makes this more than a help center.
It becomes an **RDQA operations platform**.

❓

❓

❓

❓

❓

❓

Thank you for calling me back today. The first thing to do would be to give your provider a copy of the letter that you received from Occu-Med. It would help if you pointed out that the conditions that the inquiry is being made for are on the second page of the letter. I would encourage you to provide a copy of this email as well. Please request that the provider · Be as specific as possible when addressing each bullet point in the letter to ensure a complete response is provided to each part of the inquiry. · Mention that they have reviewed the document that outlined the job functions. · Indicate whether they believe the conditions will or will not impact your ability to perform the essential job functions of the role. · Address the concerns raised while keeping in mind that their responses should revolve around the current role you are applying for rather than simply “returning to work” on a general basis. · Be sure to mention whether you do or don’t need any restrictions or reasonable accommodations due to the condition’s conditions. · Be sure to include the results of any additional testing that was performed and/or reference medical history, if indicated. I also reattached the letter to this email and highlighted the information that the provide would need to respond to. Thank you for your time and cooperation. If you have any questions, please don’t hesitate to reach out at any time. -------- Hello David, Thank you for your follow-up email. Based on page 2 of the letter that was forwarded to you by Occu-Med, you have three conditions that need to be addressed so that the medical examiner can issue the certificate. In case the language of the letter was a bit complicated, I provided a simplified explanation to you in this email. The first condition is related to the Recent pericarditis episode you experienced. to address this concern you need a letter from the cardiologist who treated the condition. The letter has to address if you have any cardiac issues that would interfere with your ability to safely drive a commercial motor vehicle. The second condition is because of your use of "Wellbutrin"; to clear this condition you need a letter from the provider who prescribed the medication to you. The letter has to address; if you have any functional limitations or limiting symptoms caused by the underlying condition that the medication was prescribed for. If the condition causes any limitations those limitations should only be addressed by the provider who issues the letter to you if they impact your ability to safely drive a commercial motor vehicle. The letter

should also state if "Wellbutrin" causes any side effects that could limit your ability to safely drive a commercial motor vehicle. The third and final condition is related to your Remote brain tumors. Per the examiner who evaluated you for your DOT exam, you need to see a neurologist and get a recent evaluation for the condition. After the neurologists conduct the evaluation they have to provide you with a letter. The letter should address the following to clear the condition. The neurologist needs to confirm that they evaluated to assess any sequelae (e.g., impairing symptoms, seizures, current lesions, etc.) that could impact your ability to safely drive a commercial motor vehicle. They must affirm that there are no concerns based on the findings of the evaluation. I truly want to help you get through the process as fast and stress-free as possible, so I wanted to provide you with a few pointers on how to best address provider letter requests. I would provide a copy of the letter you received to each provider that you visit and mention the following to them as you make your request for the letter: Please be as specific as possible when addressing each bullet point in the letter to ensure a complete response is provided to each part of the inquiry. Address the concerns raised while keeping in mind that their responses should revolve around the current role you are applying for rather than simply “returning to work” on a general basis. Be sure to include the results of any additional testing that was performed and/or reference medical history, if indicated. Be sure to mention any restrictions or reasonable accommodations that you may need for the condition. Please let me know if you have any questions or concerns. Thank you. Best Regards, ---------- Hello Jerod, Thank you for calling me back. It was a pleasure speaking to you over the phone. I want to assist you with getting through the process as fast and stress-free as possible. I wanted to provide you with a few pointers on how to best address provider letter requests. The first thing I recommend that you do is provide a copy of the letter you received from your provider. I would then mention the following. Please be as specific as possible when addressing each bullet point in the letter to ensure a complete response is provided to each part of the inquiry. Address the concerns raised while keeping in mind that their responses should revolve around the current role you are applying for rather than simply “returning to work” on a general basis. Be sure to include the results of any additional testing that was performed and/or reference medical history, if indicated. (In this case, the three blood pressure readings mentioned in bullet point A) For bullet point “A”: Per the language in the letter that was forwarded to you, " a) at least 3 clinical blood pressure readings taken over a period of at least one week demonstrating control." So that would mean visiting the clinic three days out of a week and completing a single blood pressure measurement during each visit. The visits don’t have to be consecutive, but they do have to happen all within the same week (For example either doing the visits as Monday, Tuesday, Wednesday or Monday, Wednesday, or Friday, would be fine) For bullet point “B” Mention that they have reviewed the document that outlined the job functions(attached to the letter). Provide direct responses to the following · Is there the presence of any impairing symptomatology that would be relevant to

the safe performance of the Recreation Aide/Instructor/Leader essential job functions (attached)? · Is there the presence of any target organ damage that would be relevant to the safe performance of the Recreation Aide/Instructor/Leader essential job functions (attached)? Finally, be sure to mention any restrictions or reasonable accommodations that you may need for the condition. Please let me know if I can help in any other way and if you need clarification on a certain aspect of the Thank you. ---------- As stated in my original email, Occu-Med is requesting that you provide additional information based on the findings of your evaluation. There are two RDQA conditions outlined on the second page of the letter. The first condition is “clinical documentation of a total body weight ≤ 136 kilograms”. The second condition is the “Lower back and bilateral ankle pain”. Both conditions should be addressed by your provider. The first thing to do would be to give your provider a copy of the letter that you received from Occu-Med. It would help if you pointed out that the conditions that the inquiry is being made for are on the second page of the letter. I would encourage you to provide a copy of this email as well. Please request that the provider · Be as specific as possible when addressing each bullet point in the letter to ensure a complete response is provided to each part of the inquiry. · Mention that they have reviewed the document that outlines the essential job functions. · Indicate whether they believe the conditions will or will not impact your ability to perform the essential job functions of the role. · Address the concerns raised while keeping in mind that their responses should revolve around the current role you are applying for rather than simply “returning to work” on a general basis. · Be sure to mention whether you do or don’t need any restrictions or reasonable accommodations due to the condition’s conditions. · Be sure to include the results of any additional testing that was performed and/or reference medical history, if indicated. Thank you for your time and cooperation. If you have any questions, please don’t hesitate to reach out at any time. ------- Hello Margie, It seems that you may not have received the initial email I sent, so I do apologize if the technical difficulties were on my end. I truly want to help you get through the process as fast and stress-free as possible, so I wanted to provide you with a few pointers on how to best address provider letter requests. I would provide a copy of the letter you received to each provider that you visit and mention the following to them as you make your request for the letter: · Please be as specific as possible when addressing each bullet point in the letter to ensure a complete response is provided to each part of the inquiry. · Address the concerns raised while keeping in mind that their responses should revolve around the current role you are applying for rather than simply “returning to work” on a general basis. · Be sure to include the results of any additional testing that was performed and/or reference medical history, if indicated. · Be sure to mention any restrictions or reasonable accommodations that you may need for the condition. In the case of your letter: a) the presence of any functional limitations. List and describe any functional limitations that may interfere with the individual’s ability to perform the necessary job functions of the role. b) whether the performance of the essential Mealsite Coordinator job functions (attached)

places the individual at a significantly increased risk of aggravation or exacerbation of the condition. Direct response on whether the individual's current health status and the specific condition listed in the letter would be at an increased risk of aggravation or exacerbation if performing the job functions of the role. If not, please affirm that that is the case. c) any current side effects experienced secondary to the prescribed treatment regimen; and, Direct response to whether the individual is experiencing any side effects due to the treatment that they are receiving for the condition. If so, please confirm if the side effects could impact the individuals' ability to perform the necessary job functions of the role. d) any restrictions or accommodations recommended to permit the safe and consistent performance of essential job tasks in light of the underlying condition and its associated treatment regimen. Direct response to whether the individual requires any reasonable accommodations or has any restrictions. Please let me know if you have any questions or concerns Thank you. Best Regards, Alex Ayvazian --------- Hello Tyler, Thank you for your kind response. I want to be as helpful as possible to you during this process, so I wanted to provide some pointer on the requested letter. It's very common for physicians to be unaware on how to address these types of requests and provide a general vague letter when one is requested of them. This unfortunately creates a back-and-forth situation for the applicant as they end up needing the letter to be amended to include additional information. I would provide a copy of the letter you received to your provider and make sure to mention the following (or provide a copy of this email). Please request that the provider Be as specific as possible when addressing each bullet point in the letter to ensure a complete response is provided to each part of the inquiry. Mention that they have reviewed the document that outlined the job functions. Address the concerns raised while keeping in mind that their responses should revolve around the current role you are applying for rather than simply "returning to work" on a general basis. Be sure to include results of any additional testing that was performed and/or reference medical history, if indicated. Be sure to mention any restrictions or reasonable accommodations that you may need for the condition. Thank you for your time and cooperation. If you have any questions, please don't hesitate to reach out at any time. ------- Hello Jose, I am following up on your email. Our subject matter experts reviewed the information you provided and based on the results of your medical evaluation they are requesting that you follow up with a medical provider and address the concerns outlined on page 2 of the letter. There are three condition's that need to be addressed and Vectrus has given you a deadline to do so until 8/14. The first thing to do would be to give your provider a copy of the letter that you received from Occu-Med. It would help if you pointed out that the conditions that the inquiry is being made for are on the second page of the letter. I would encourage you to provide a copy of this email as well. Please request that the provider · Be as specific as possible when addressing each bullet point in the letter to ensure a complete response is provided to each part of the inquiry. · Mention that they have reviewed the document that outlined the job functions. · Indicate whether they believe the conditions will or will not

impact your ability to perform the essential job functions of the role. · Address the concerns raised while keeping in mind that their responses should revolve around the current role you are applying for rather than simply “returning to work” on a general basis. · Be sure to mention whether you do or don’t need any restrictions or reasonable accommodations due to the condition’s conditions. · Be sure to include the results of any additional testing that was performed and/or reference medical history, if indicated. Thank you for your time and cooperation. If you have any questions, please don’t hesitate to reach out at any time. ------- Hello Robert, Thank you for calling me back today. The first thing to do would be to give your provider a copy of the letter that you received from Occu-Med. It would help if you pointed out that the conditions that the inquiry is being made for are on the second page of the letter. I would encourage that you provide a copy of this email as well. Please request that the provider · Be as specific as possible when addressing each bullet point in the letter to ensure a complete response is provided to each part of the inquiry. · Mention that they have reviewed the document that outlined the job functions. · Indicate whether they believe the conditions will or will not impact your ability to perform the essential job functions of the role. · Address the concerns raised while keeping in mind that their responses should revolve around the current role you are applying for rather than simply “returning to work” on a general basis. · Be sure to mention whether you do or don’t need any restrictions or reasonable accommodations due to the condition’s conditions. · Be sure to include results of any additional testing that was performed and/or reference medical history, if indicated. Thank you for your time and cooperation. If you have any questions, please don’t hesitate to reach out at any time. ------- Best Regards, Hello Matthew, Yes, that would be perfect. I have added some guidance to this email for clarification on how the form must be filled out so the provide can avoid making commonly made mistakes. According to MOD 17 guidelines, to be deemed deployable, your dental health must be in a condition where you will not require evaluation or treatment for any oral condition that could result in a dental emergency within 12 months of deployment. Your dental provider needs to complete the DD2813 form and classify you as either Category 1 (box 1) or Category 2 (box 2). For Category 2, the provider should administer the minimum necessary treatment to ensure you are unlikely to experience a dental emergency. Category 2 indicates that although further dental treatment might be needed, the risk of a dental emergency is very low. Please note that only one box should be checked off by the provider. A blank copy of the dental exam form was attached to the letter I sent you earlier. I reattached the letter to this email. If you have any other questions or concerns, feel free to let me know. Best regards, ------- Hello Josh, Thank you for following up. Please find the information I was referencing in the below email. The first thing to do would be to give your provider a copy of the letter that you received from Occu-Med. It would help if you pointed out that the conditions that the inquiry is being made for are on the second page of the letter. I would encourage you to provide a copy of this email as well. Please request that the provider Be as specific as possible when

addressing the letter to ensure a complete response is provided to each part of the inquiry. Mention that they have reviewed the document that outlined the job functions. Indicate whether they believe the conditions will or will not impact your ability to perform the essential job functions of the role. Address the concerns raised while keeping in mind that their responses should revolve around the current role you are applying for rather than simply “returning to work” on a general basis. Be sure to mention whether you do or don’t need any restrictions or reasonable accommodations due to the condition’s conditions. Be sure to include the results of any additional testing that was performed and/or reference medical history, if indicated. Thank you for your time and cooperation. If you have any questions, please don’t hesitate to reach out at any time. ------- Best Regards, Hello Luke, Thank you for taking my call today. The first thing to do would be to give your provider a copy of the letter that you received from Occu-Med. It would help if you pointed out that the conditions that the inquiry is being made for are on the second page of the letter. I would encourage you to provide a copy of this email as well. Please request that the provider · Be as specific as possible when addressing each bullet point in the letter to ensure a complete response is provided to each part of the inquiry. · Mention that they have reviewed the document that outlined the job functions. · Indicate whether they believe the conditions will or will not impact your ability to perform the essential job functions of the role. · Address the concerns raised while keeping in mind that their responses should revolve around the current role you are applying for rather than simply “returning to work” on a general basis. · Be sure to mention whether you do or don’t need any restrictions or reasonable accommodations due to the condition’s conditions. · Be sure to include the results of any additional testing that was performed and/or reference medical history, if indicated. Thank you for your time and cooperation. If you have any questions, please don’t hesitate to reach out at any time. ------- Best Regards, Alex Ayvazian EXAMQA, Analyst Team Bailey 2121 West Bullard Avenue Hello Benson, I hope you are doing well. I am following up regarding the information you submitted for your RDQA condition. I want to provide you with some guidance on how to best address this request so that you can get everything that Is needed before your deadline expires. I attached a job description for your role to this email (your provider should revise the highlighted portion of the job description before drafting the statement / letter for you). I have attached the letter and highlighted the specific part that your provider needs to read respond to in the form of a statement / letter. The first thing to do would be to give your provider a copy of the letter that you received from Occu-Med. It would help if you pointed out that the conditions that the inquiry is being made for are on the second page of the letter. I would encourage you to provide a copy of this email as well. Please request that the provider Be as specific as possible when addressing each bullet point in the letter to ensure a complete response is provided to each part of the inquiry. Mention that they have reviewed the document that outlined the job functions. Indicate whether they believe the conditions will or will not impact your ability to perform the essential job functions

of the role. Address the concerns raised while keeping in mind that their responses should revolve around the current role you are applying for rather than simply “returning to work” on a general basis. Be sure to mention whether you do or don’t need any restrictions or reasonable accommodations due to the condition’s conditions. Be sure to include the results of any additional testing that was performed and/or reference medical history, if indicated. Thank you for your time and cooperation. If you have any questions, please don’t hesitate to reach out at any time. ------- Best Regards, ------ Hello Reginald, I am following up on the information you submitted to Occu-Med for review. Our subject matter experts have reviewed the information, and it seems that some of their concerns have not been addressed. Per our experts, The questions that was raised in the letter you received should be answered by the provider. Occu-Med needs to know about the stability and expectations for the next year. In the letter you received the following question were posed by our experts, “a) current status of the condition, including the presence of any functional limitations (i.e. scotomas); b) stability of the condition (i.e. duration on the current treatment regimen, stability of ocular pressures, etc.); c) any ongoing monitoring that is required over the proposed deployment term; d) whether prescribed medications involve any special storage (i.e. refrigeration); and, e) whether the environmental conditions in Kuwait pose a significantly increased risk of aggravating or exacerbating the condition. If yes, please outline any mitigating restrictions that are recommended.” Kindly ask the provider to respond to each point as if they were questions that need complete responses. Thank you. ------- Best Regards, Alex Ayvazian EXAMQA, Analyst Team Bailey 2121 West Bullard Avenue Fresno, CA 93711 for Mario Yoli Hello Mario, I hope you are doing well. I am following up on the conversation we had yesterday regarding the RDQA letter. If You recall, I was going to provide you with some guidance on how to best address this request. The first thing to do would be to give your provider a copy of the letter that you received from Occu-Med. It would help if you pointed out that the condition that the inquiry is being made for is on the second page of the letter. I encourage you to provide a copy of this email to the provider as well. Please request that the provider · Be as specific as possible when addressing each bullet point in the letter to ensure a complete response is provided to each part of the inquiry. · Mention that they have reviewed the document that outlined the job functions. · Indicate whether they believe the conditions will or will not impact your ability to perform the essential job functions of the role. · Address the concerns raised while keeping in mind that their responses should revolve around the current role you are applying for rather than simply “returning to work” on a general basis. · Be sure to mention whether you do or don’t need any restrictions or reasonable accommodations due to the condition’s conditions. · Be sure to include the results of any additional testing that was performed and/or reference medical history, if indicated. Guidance specific to your condition: The first step would be to get the 3 clinical blood pressure readings completed (all three of the readings can be completed during a single visit to your providers clinic). The second step would be to obtain a statement

from the provider that addresses your elevated hypertension and responds to point “b” on page two of the letter. Did the provider the presence of any impairing symptomatology or target organ damage that would be relevant to the employee's safe performance of the Aircraft Mechanic essential job functions? Does the provider believe that you can safely perform the essential job functions of your current role with or without reasonable accommodation? Please follow the general guidance provided at the beginning of the email when addressing these questions. Thank you for your time and cooperation. If you have any questions, please don’t hesitate to reach out at any time. -- Best Regards, Alex Ayvazian EXAMQA, Analyst Team Morgan 2121 West Bullard Avenue Fresno, CA 93711 Hello Nathaniel, Thank you for your follow-up. I’m happy to assist with requesting a deadline extension. Please confirm the new deadline date that works for you. I will do my best to help further explain what is being requested and provide some guidance. I attached a job description for your role to this email (your provider should revise the highlighted portion of the job description before drafting the statement / letter for you). I have attached the letter and highlighted the specific part that your provider needs to read respond to in the form of a statement / letter. The first thing to do would be to give your provider a copy of the letter that you received from Occu-Med. It would help if you pointed out that the conditions that the inquiry is being made for are on the second page of the letter. I encourage you to provide a copy of this email as well. Please request that the provider Be as specific as possible when addressing each bullet point in the letter to ensure a complete response is provided to each part of the inquiry. Mention that they have reviewed the document that outlined the job functions. Indicate whether they believe the conditions will or will not impact your ability to perform the essential job functions of the role. Address the concerns raised while keeping in mind that their responses should revolve around the current role you are applying for rather than simply “returning to work” on a general basis. Be sure to mention whether you do or don’t need any restrictions or reasonable accommodations due to the condition’s conditions. Be sure to include the results of any additional testing that was performed and/or reference medical history, if indicated. Guidance Specific to your conditions: The first condition is regarding your EKG result we received from the clinic that performed your evaluation. The examiner noted an abnormal finding. To address this condition, you would need to: Undergo a cardiological evaluation. The evaluating physician can then determine if he needs you to undergo additional testing. Upon completing the evaluation, the physician should issue a statement addressing any ongoing treatment/monitoring that you may need and the speak to your ability to deploy to and work in Jordan as a Recreational Services Associate (Any work restrictions should also be outlined by the physician, if determined to be necessary to allow safe performance of the essential job functions). When the physician drafts the statement tell them to follow the guidance I mention above (Paragraph that starts with “Please request that the provider”) You blood pressure readings were also elevated. This means that the physician would need to evaluate your blood pressure and complete the form attached to this email. The must show

that your blood pressure is controlled. If you end up getting diagnosed with hypertension and needing treatment, the provider will then need to address the following paragraph on page 2 paragraph 2 of the letter. “[Note: In the event that the individual is diagnosed with hypertension and begins treatment, documentation of 30 days stability must be documented in order to request a medical waiver for deployment. Such documentation should include updated blood pressure readings at the end of the 30-day period and a statement from the treating physician addressing whether the individual has tolerated hypertension medications without adverse side effects.]” Please let me know if you have any questions or concerns. Thank you! -- Best Regards, ------0 Hello Yessica, Thank you for calling me back today. The first thing to do would be to give your provider a copy of the letter that you received from Occu-Med. It would help if you pointed out that the conditions that the inquiry is being made for are on the second page of the letter. I would encourage you to provide a copy of this email as well. Please request that the provider · Be as specific as possible when addressing each bullet point in the letter to ensure a complete response is provided to each part of the inquiry. · Mention that they have reviewed the document that outlined the job functions. · Indicate whether they believe the conditions will or will not impact your ability to perform the essential job functions of the role. · Address the concerns raised while keeping in mind that their responses should revolve around the current role you are applying for rather than simply “returning to work” on a general basis. · Be sure to mention whether you do or don’t need any restrictions or reasonable accommodations due to the condition’s conditions. · Be sure to include the results of any additional testing that was performed and/or reference medical history, if indicated. Thank you for your time and cooperation. If you have any questions, please don’t hesitate to reach out at any time. ------- Best Regards, Alex Ayvazian EXAMQA, Analyst Team Bailey 2121 West Bullard Avenue --------- information. I hope you are doing well. I am following up on the information you submitted to Occu-Med for review. Our subject matter experts have reviewed the information and provided the following feedback: Regarding the first condition, "Obstructive Sleep Apnea": Unfortunately, the CPAP compliance report you provided does not meet the MOD 17 deployment guidelines. As mentioned in the letter sent to you, individuals need to use the CPAP for more than 4 hours per night. You need to submit another compliance report for the past 30 days showing that you used the CPAP for at least 4 hours per night for at least 27 out of the 30 days. If this is unattainable, please provide a letter from your physician explaining why this is not possible. Regarding the second condition, "Use of Nebido and choriomon": While the information provided was helpful, some aspects of the inquiry were not addressed. The provider needs to indicate if there are any special handling requirements for the medication you are prescribed. Please let me know if you have any questions or concerns. Best regards, ------- Best Regards, Alex Ayvazian EXAMQA, Analyst Team Bailey 2121 West Bullard Avenue ------ Subject: Vectrus: Follow up on requested additional information. I hope you are doing well. I am following up on the information you submitted to

Occu-Med for review. Our subject matter experts have reviewed the information and provided the following feedback: Regarding the first condition, "Obstructive Sleep Apnea": Unfortunately, the CPAP compliance report you provided does not meet the MOD 17 deployment guidelines. As mentioned in the letter sent to you, individuals need to use the CPAP for more than 4 hours per night. You need to submit another compliance report for the past 30 days showing that you used the CPAP for at least 4 hours per night for at least 27 out of the 30 days. If this is unattainable, please provide a letter from your physician explaining why this is not possible. Regarding the second condition, "Use of Nebido and choriomon": While the information provided was helpful, some aspects of the inquiry were not addressed. The provider needs to indicate if there are any special handling requirements for the medication you are prescribed. Please let me know if you have any questions or concerns. Best regards, ------- Best Regards, Alex Ayvazian EXAMQA, Analyst Team Bailey ---------- Hello Damian, Thank you for reaching out with your request. I submitted the deadline extension request to V2X and will get back to you as soon as I hear from them about the matter. ---------- I will do my best to help further explain what is being requested and provide some guidance. I attached a job description for your role to this email (your provider should revise the highlighted portion of the job description before drafting the statement / letter for you). I have attached the letter and highlighted the specific part that your provider needs to read respond to in the form of a statement / letter. The first thing to do would be to give your provider a copy of the letter that you received from Occu- Med. It would help if you pointed out that the conditions that the inquiry is being made for are on the second page of the letter. I encourage you to provide a copy of this email as well. General guidance for the provider issuing a statement / letter: Please request that the provider Be as specific as possible when addressing each bullet point in the letter to ensure a complete response is provided to each part of the inquiry. Mention that they have reviewed the document that outlined the job functions. Indicate whether they believe the conditions will or will not impact your ability to perform the essential job functions of the role. Address the concerns raised while keeping in mind that their responses should revolve around the current role you are applying for rather than simply “returning to work” on a general basis. Be sure to mention whether you do or don’t need any restrictions or reasonable accommodations due to the condition’s conditions. Be sure to include the results of any additional testing that was performed and/or reference medical history, if indicated. ---------- Guidance Specific to your conditions: The first condition is regarding Your blood pressure readings we receiceved from the clinic. The physician who would need to address this does not have to be a cardiologist (it can also be your primary care provider, or any other provider of your choosing). The physician should evaluate your blood presure. Upon doing so they should respond to the following questions in the form of a letter / statement: Does the physician believe that there is the presence of any end organ damage caused by your hypertension? Does the physician believe you experience any hypertension-related symptomatology,

complications experienced or associated sequelae? Does the physician recommend follow-up or ongoing monitoring expected over the next 12 months? If so, please specify. Additionally, the physican should include the following in the statement: documentation of control and stability of the condition which can be done by providing at least 3 serial blood pressure measurements taken in a clinical setting (e.g., ≤140/90). complete list of prescribed medications. Explanation on whether prescribed medications have produced any impairing side effects. ----- The second condtion is regarding your Elevated atherosclerotic cardiovascular disease ("ASCVD") risk percentage. The elevated result requires that you follow up with a cardiologist to undergo cardiovascular evaluation. This can be any provider of your choosing and it does not have the be a cardiologist you have seen before. The Cardiovascular evaluation should incude either of the following tests based on the evaluating providers determination: stress echocardiography graded exercise stress test with myocardial perfusion scintigraphy (SPECT scan) Once the evaluation / testing is completed, the provider should issue a statement / letter addressibg the following (please ask the provider to read the section where I have provided general guidance for their convince): What were the findings of the evaluation and do the findinsg reveal any cardiac condition that would either indicate treatment (if yes, please detail) or interfere with the ability to safely deploy to and work in Marshall Islands as a/an Fire Chief. Thank you for your time and cooperation. If you have any questions, please don’t hesitate to reach out at any time. -- Best Regards, Alex Ayvazian --------- Hello Damian, Our subject matter experts reviewed the information you provided, and I followed up with them to obtain feedback from them. It seems that we are still missing information for the following conditions: “Hypertensive urgency requires evaluation by a qualified physician and statement addressing: a) the presence of any end organ damage; b) documentation of control and stability of the condition as evidenced by submission of at least 3 serial blood pressure measurements taken in a clinical setting (e.g., ≤140/90); c) complete list of prescribed medications and whether prescribed medications have produced any impairing side effects; d) any hypertension-related symptomatology, complications experienced or associated sequelae; and, e) any recommended follow-up or ongoing monitoring expected over the next 12 months. [Note: Hypertensive urgency requires a medical waiver in order to deploy. Accordingly, all associated medical records, investigative testing, etc. should be provided to support such a request.] “ “Elevated atherosclerotic cardiovascular disease ("ASCVD") risk percentage requires cardiological evaluation to include either stress echocardiography or graded exercise stress test with myocardial perfusion scintigraphy (SPECT scan) as determined by the evaluating physician. A statement must also be provided from the evaluating physician documenting whether the findings of the evaluation reveal any cardiac condition that would either indicate treatment (if yes, please detail) or interfere with the ability to safely deploy to and work in Marshall Islands as a/an Fire Chief. [Note: The deployment authority does not permit waivers without recent functional cardiological testing provided in response to elevated ASCVD risk percentage. If

no functional cardiological testing is provided, the evaluating or treating cardiologist should provide the supporting rationale.]" The hypertension related information is still needed for the final recommendation to be issued on your behalf. Additionally, the waiver authority will ask for the information out line in both conditions. If we don't send the above outlined information to the waiver authority, they will deny the waiver application. If you are having a hard time understanding what is being requested, please reach out to my so that I can help you by possibly simplifying the language. We will need to apply for a deadline extension to keep your case active with V2X. Would an extension until 9/15/24 be enough time for you? Thank you! -- Best Regards, ------- Hello Ryan, The dental provider made an error on your form. According to MOD 17 guidelines, to be deemed deployable, your dental health must be in a condition where you will not require evaluation or treatment for any oral condition that could result in a dental emergency within 12 months of deployment. Your dental provider needs to complete the DD2813 form and classify you as either Category 1 (box 1) or Category 2 (box 2) (Please see form attached named as "Guide") For Category 2, the provider should administer the minimum necessary treatment to ensure you are unlikely to experience a dental emergency. Category 2 indicates that although further dental treatment might be needed, the risk of a dental emergency is very low. Please note that only one box should be checked off by the provider. If you have any other questions or concerns, feel free to let me know. -- Best Regards, Alex Ayvazian EXAMQA, Analyst Team Morgan 2121 West Bullard Avenue Fresno, CA 93711 -------- Hello Keith, I hope you are doing well. I want to provide you with some guidance on how to best address this request so that you can get everything that Is needed before your deadline expires. I attached a job description for your role to this email (your provider should revise the highlighted portion of the job description before drafting the statement / letter for you). I have attached the letter your provider needs to read and respond to in the form of a statement / letter. The first thing to do would be to give your provider a copy of the letter that you received from Occu-Med (attached). It would help if you pointed out that the conditions that the inquiry is being made for are on the second page of the letter. I would encourage you to provide a copy of this email as well. Please request that the provider · Be as specific as possible when addressing each bullet point in the letter to ensure a complete response is provided to each part of the inquiry. · Mention that they have reviewed the document that outlined the job functions. · Indicate whether they believe the conditions will or will not impact your ability to perform the essential job functions of the role. · Address the concerns raised while keeping in mind that their responses should revolve around the current role you are applying for rather than simply "returning to work" on a general basis. · Be sure to mention whether you do or don't need any restrictions or reasonable accommodations due to the condition's conditions. · Be sure to include the results of any additional testing that was performed and/or reference medical history, if indicated. Thank you for your time and cooperation. If you have any questions, please don't hesitate to reach out at any time. -- Best Regards, Alex Ayvazian EXAMQA, Analyst Team Morgan 2121

West Bullard Avenue ---------- Hello Thomas, Thank you for reaching out with that important question. What seems to be missing in the section you highlighted are the following: 3-day series of blood pressure readings after stabilization of the treatment regimen/condition. Statement from the treating physician confirming that the employee has tolerated hypertension medications without adverse side effects. The best way to address this is to ask your provider if they believe the medication has stabilized your blood pressure. If the provider agrees that the effects of the medication have taken place, you can proceed with the 3 blood pressure readings. Each reading should be done on a separate day, but all three readings must be completed within the same week. Once you have the readings, the provider can then draft the statement confirming that you tolerated the hypertension medications without adverse side effects. They should also mention the values of the readings to supoort their conclusion. It would also be helpful if they note that you can perform the essential job functions of your position with or without any restrictions or reasonable accommodations. Please let me know if you have any other questions or concerns that I can address. Thank you. -- Best Regards, Alex Ayvazian EXAMQA, Analyst Team Morgan 2121 West Bullard Avenue Fresno, CA 93711 T: (559) 435-2800 x135 ---------- bject: Vectrus: Follow up on RDQA Hello Earl, I'm following up on the information we received from the clinic regarding your diabetes. After review by our subject matter experts, it has been determined that while some aspects of your diabetes were addressed, the inquiry made in the original letter was not fully covered. I believe this matter would be best addressed by your primary care provider. In the following email, I am going to provide some guidance to help you address this process. The first thing to do would be to give your provider a copy of the letter that you received from Occu-Med. It would help if you pointed out that the conditions that the inquiry is being made for are on the second page of the letter. I encourage you to provide a copy of this email as well. General guidance for the provider issuing a statement / letter: Please request that the provider Be as specific as possible when addressing each bullet point in the letter to ensure a complete response is provided to each part of the inquiry. Mention that they have reviewed the document that outlined the job functions. Indicate whether they believe the conditions will or will not impact your ability to perform the essential job functions of the role. Address the concerns raised while keeping in mind that their responses should revolve around the current role you are applying for rather than simply "returning to work" on a general basis. Be sure to mention whether you do or don't need any restrictions or reasonable accommodations due to the condition's conditions. Be sure to include the results of any additional testing that was performed and/or reference medical history, if indicated. Guidance Specific to your conditions: The condition in question relates to your diabetes and the recent elevated A1C lab result. The provider should review the attached A1C lab result and address the following in a statement or letter: Is the result indicative of an underlying medical condition? If so, please include the diagnosis. Does the individual show any target organ damage, related complications, or impairing symptoms associated with this

condition? Was any treatment initiated? If yes, kindly provide a detailed list of treatments. Will ongoing monitoring be required throughout the 1-year deployment period? (If yes, please specify how often the individual would need to condcut follow up visits) Additionally, the provider should perform a diabetic foot exam and nutrition counseling. The findings should either be included in the attached statement or noted in the letter. Thank you for your time and cooperation. If you have any questions, please don't hesitate to reach out at any time. -- Best Regards, Alex Ayvazian EXAMQA, Analyst Team Morgan

Thought for a couple of seconds
Alex, **THIS is exactly the evidence you need for the pitch.** What you pasted proves the entire business case:
Occu-Med is repeatedly spending analyst time translating dense RDQA language into plain-English instructions, reminding applicants to give the provider the letter, explaining what the provider must address, clarifying job-function language, correcting incomplete provider responses, chasing missing documentation, and asking for deadline extensions.
That means the portal should not just be a "resource center."
It should be an **RDQA Translation + Guidance + Document Completion System**.

The Core Pitch to Andy

Occu-Med can reduce Exam QA workload by turning repetitive RDQA explanation into a guided digital workflow that tells applicants exactly what their provider needs to address, checks whether the response is likely complete before submission, and gives clients better visibility into why cases are delayed.
This creates three wins:

Win	Meaning
Cost control	Fewer repetitive calls/emails from EQA analysts
Faster case resolution	Applicants submit better documentation the first time
Revenue opportunity	Clients can be charged for enhanced RDQA support, analytics, and applicant guidance

The Smartest Portal Ideas Based on Your Real Examples

1. RDQA Letter Translator

This is the biggest one.
Applicants receive language like:
"History of diabetes mellitus with most recent hgbA1c of 9.84% requires documentation of improved control..."
Most people do not understand that. The portal should translate it into:

What this means

Your exam showed an elevated A1c result related to diabetes. Occu-Med needs updated medical information showing whether the condition is controlled and whether it could affect your ability to safely perform your job duties.

What your provider needs to address

Your provider should answer:

1. What is your current diabetes status?
2. Are there any complications, such as kidney, nerve, eye, heart, or circulation problems?
3. Is your diabetes stable?
4. What medications or treatment are you currently using?
5. Have you had any recent high or low blood sugar episodes?
6. Will you need monitoring during the deployment period?
7. Do you need restrictions or accommodations?

That is literally the work you are already doing manually in those emails.
The portal should do it automatically.

2. “Provider Question Builder”

Instead of telling applicants, “have your provider address each bullet point,” the portal should generate a clean provider-facing question sheet.
Example:

Provider Response Form: Hypertension

Please respond to each item below:

Question	Provider Response
Is there evidence of end-organ damage?	
Is the condition controlled and stable?	
Please list at least 3 clinical blood pressure readings.	
What medications are prescribed?	
Are there any impairing medication side effects?	
Are there symptoms, complications, or sequelae?	
Is ongoing monitoring recommended over the next 12 months?	
Are restrictions or accommodations needed?	

Can the individual perform the essential job functions with or without accommodation?	

This could be generated for every RDQA condition.
That would massively reduce vague provider notes like:
"Patient is cleared to work."
Because the provider would be forced to respond point-by-point.

3. Condition-Specific RDQA Playbooks

The portal should have condition modules based on the actual patterns you showed.

High-Value Initial Modules

RDQA Condition	Portal Guidance Should Include
Elevated blood pressure	3 clinical readings, medication tolerance, end-organ damage, symptoms, monitoring
Hypertensive urgency	Waiver warning, clinical control, serial BP readings, meds, side effects, 12-month monitoring
Diabetes / elevated A1c	Updated A1c, complications, treatment regimen, hypo/ hyperglycemic events, monitoring
Elevated ASCVD risk	Cardiology evaluation, stress echo or SPECT, provider rationale, deployment impact
Depression / behavioral health	Current stability, baseline symptoms, residual symptoms, lethality, treatment history, 90-day medication stability
Sleep apnea	CPAP compliance, 30-day usage, 4+ hours per night, 27/30 days, physician explanation if not attainable
Orthopedic pain	Functional limitations, risk of aggravation, treatment needs, restrictions/accommodations
Dental abnormality	DD2813 Category 1 or 2, only one box checked, no anticipated emergency within 12 months

Abnormal labs	Provider comment on all out-of-range values, repeat/additional testing if needed
Medication use	Purpose of medication, side effects, special handling/storage, functional limitations
Neurological history	Seizures, lesions, sequelae, impairing symptoms, neurologist evaluation
Cardiac abnormality	Cardiology evaluation, treatment needs, monitoring, ability to deploy/work safely

This becomes an internal knowledge base and applicant-facing guidance system.

4. “What Should I Bring to My Doctor?” Packet Generator

Applicant selects their condition and role.
The portal generates a downloadable packet:

Provider Visit Packet

Includes:

1. Copy of RDQA request language
2. Plain-English explanation
3. Provider question sheet
4. Essential job functions
5. Testing requirements
6. Common mistakes to avoid
7. Submission instructions
8. Deadline reminder

This replaces half of the emails you pasted.
For example, for hypertension, the packet would say:
Please bring this packet to your provider. The provider should review your essential job functions and issue a statement addressing blood pressure control, medication side effects, symptoms, complications, and whether restrictions or accommodations are needed.

5. “Incomplete Letter Detector”

This is a killer feature.
The portal should let applicants upload their provider letter before final submission and run a checklist scan.
Example:

Provider Letter Review

Uploaded document appears to include:

Requirement	Status
Provider name	Found
Date	Found
Diagnosis/status	Found
Medication list	Missing
Restrictions/accommodations	Missing
Essential job functions mentioned	Missing
Monitoring plan	Missing

Then it tells the applicant:
This document may not be complete. Your provider did not appear to address restrictions/accommodations or whether the condition impacts the essential job functions.
Even if the first version is not AI-powered, EQA can manually mark missing items in the portal and the system generates the follow-up automatically.

6. Applicant "One Next Step" Dashboard

Applicants get overwhelmed because RDQA letters are dense.
The portal should not show everything all at once. It should show:

Your Next Step

Give the attached provider guidance packet to your physician and request a written statement addressing each question.

Then:

Your Required Items

Item	Status
Provider statement	Needed
3 blood pressure readings	Needed
Medication list	Needed
Restrictions/accommodation statement	Needed

This would reduce calls like:
"What am I supposed to do?"
"What does this mean?"
"Do I need a specialist?"
"Can my PCP do this?"
"What should the letter say?"

7. Smart Specialist Routing

The portal should tell the applicant what kind of provider is likely needed.
Examples:

Condition	Likely Provider
Elevated BP	Primary care provider may be acceptable
Hypertensive urgency	Qualified physician; PCP or cardiologist depending on case
ASCVD risk	Cardiologist
Pericarditis	Treating cardiologist
Brain tumor history	Neurologist
Depression	Treating behavioral health provider / psychiatrist / psychologist depending on requirement
Dental abnormality	Dentist
Diabetes	Primary care provider or endocrinologist
Orthopedic pain	PCP, orthopedist, or treating provider
Sleep apnea	Sleep medicine provider, pulmonologist, or treating physician

This prevents applicants from wasting time with the wrong provider.

8. “Common Mistakes” Warnings

Your emails show the same mistakes repeatedly.
The portal should warn applicants before they submit anything.

Common Provider Letter Mistakes

- Provider says only “cleared to work”
- Provider does not mention the job functions
- Provider does not address restrictions or accommodations
- Provider does not answer each bullet point
- Provider does not include recent test results
- Provider does not mention medication side effects
- Provider does not explain ongoing monitoring needs
- Dental provider checks the wrong DD2813 category
- Dental provider checks more than one box
- CPAP report does not meet the 30-day / 27-day threshold
- Blood pressure readings are not taken in a clinical setting
- Blood pressure readings are not taken over the required period
- Letter does not include provider name, signature, date, or credentials

This alone would save time.

9. Deadline Extension Assistant

Several of your examples involve deadlines and extension requests.
The portal should have:

Need More Time?

Applicant clicks a button and answers:

- Why do you need an extension?
- When is your provider appointment?
- What date do you expect to submit records?
- Upload proof of appointment, if available
- Requested new deadline

Then EQA receives a clean internal request:
Applicant requests extension to 09/15/2024 due to pending cardiology appointment on 09/10/2024.
This reduces messy email chains.

10. Internal EQA "Smart Reply Generator"

This would be huge for analysts.
EQA chooses:

- Client
- Applicant name
- Condition
- Missing item
- Tone
- Deadline
- Whether job description is attached
- Whether provider letter is incomplete

The system generates the email.
Example options:

Scenario	Generated Email
Initial RDQA explanation	Plain-English applicant guidance
Provider letter incomplete	Missing items follow-up
Deadline extension	Extension request language
Dental form error	DD2813 correction email
CPAP noncompliance	Sleep apnea follow-up
Hypertension clarification	3-reading explanation
Provider did not address job duties	Job-function clarification

This turns your best emails into reusable company templates.

11. Job Function Confirmation Module

A repeated issue in your examples is that the provider needs to review the essential job functions.

The portal should force this step.
Provider packet should include:
Please confirm that you reviewed the attached essential job functions before issuing your statement.
Then the provider response form includes:

Required Statement	Checkbox
I reviewed the essential job functions for this position.	☐
I considered whether the condition impacts safe performance of those duties.	☐
I addressed whether restrictions or accommodations are needed.	☐

This directly targets one of the biggest reasons provider notes are incomplete.

12. Client-Specific Rules Engine

Different clients and standards may have different needs.
The portal should support client-specific rule sets.
Examples:

Client / Standard	Portal Behavior
V2X / deployment	Includes deployment language, host-nation, essential job functions, waiver reminders
DOT/FMCSA	Uses commercial motor vehicle language
MOD 17	Includes deployment-specific requirements
CENTCOM	Includes weight/BMI/body fat documentation language
CDOT / California clients	Uses client-specific language and avoids deployment references

This makes the portal scalable across Occu-Med's client base.

13. RDQA Readiness Score

This would impress Andy because it turns chaos into a measurable workflow.
Example:

Case Readiness: 68%

Requirement	Status
Provider statement received	Complete

Job functions addressed	Missing
Restrictions/accommodations addressed	Missing
Recent test results included	Complete
Monitoring plan included	Missing

Result:
Not ready for SME review.
This helps avoid wasting SME time on incomplete submissions.

14. “Why This Matters” Applicant Education

Applicants do not understand why Occu-Med keeps asking for more information.
The portal should explain:
A provider letter must address the specific concerns listed in your RDQA request.
A general statement saying you are “cleared” may not be enough because Occu-Med must evaluate the information against job duties, client requirements, and applicable medical guidelines.
This is important because it reduces frustration.

15. Provider-Facing Mini Portal

This is an advanced idea but extremely smart.
Instead of relying on applicants to explain everything, providers could access a secure link with:

- The exact condition to address
- Job functions
- Required questions
- Upload button
- Provider attestation
- Signature/date field

The provider sees:

Occu-Med is requesting medical documentation only.

You are not being asked to make an employment decision. Please document the individual’s current medical status, treatment, functional limitations, restrictions/accommodations, and whether the condition may impact the listed job functions.
That single clarification could prevent vague or legally awkward provider notes.

16. “Plain-English RDQA Letter” Auto-Attachment

Every RDQA letter could be paired with a simplified cover page.
Example:

Your RDQA Request in Plain English

You are being asked to provide additional information for:

1. Elevated blood pressure
2. Diabetes / elevated A1c

3. Dental status

For each condition, here is what your provider needs to address…
This reduces the need for EQA analysts to send separate explanation emails later.

17. Dental Form Error Prevention Tool

Dental is perfect for a guided tool because the errors are predictable.

DD2813 Dental Guidance

The portal should explain:

- Provider must complete DD Form 2813
- Category 1 or Category 2 is generally needed
- Only one category box should be checked
- Category 2 means dental treatment may be needed but no emergency is expected within 12 months
- If Category 3 is selected, treatment or re-evaluation may be needed
- The provider should sign/date the form

This would reduce repeated dental correction emails.

18. Sleep Apnea Compliance Calculator

For OSA cases:
Applicant uploads or enters CPAP usage.
The portal checks:

- Is the report for the most recent 30 days?
- Were there at least 27 days with 4+ hours of use?
- Is the applicant's name visible?
- Is the date range visible?
- Does the report show usage hours?

Then it says:
This report appears to meet the minimum usage threshold.
or
This report may not meet the required threshold. Please obtain a new 30-day report or ask your physician to explain why this is not attainable.
This would be insanely useful.

19. Blood Pressure Reading Tracker

For hypertension cases:
Applicant enters clinical readings:

Date	Reading	Location
Monday	138/86	Clinic
Wednesday	136/84	Clinic
Friday	132/82	Clinic

Portal checks:

- Are there at least 3 readings?

- Are they from a clinical setting?
- Were they completed within the required timeframe?
- Are they under the threshold?
- Is medication stability needed?

Then it generates a summary for the provider.

20. “Provider Must Address” Highlighting Tool

You currently manually highlight the letter and tell applicants what to show providers.

The portal should let EQA paste or upload the RDQA language and mark:

- Condition
- Required questions
- Specialist type
- Required testing
- Deadline
- Job title

The portal then generates:

- Applicant explanation
- Provider checklist
- Email template
- Upload checklist
- Client status summary

This is where AI could help a lot.

21. Client Dashboard: Why Cases Are Delayed

This is how Occu-Med can charge more.

Clients should be able to see:

Delay Reason	Number of Cases
Awaiting applicant records	18
Provider letter incomplete	9
Awaiting specialist appointment	6
Awaiting SME review	4
Awaiting client deadline extension	3

This makes Occu-Med look organized and transparent.

It also proves value.

Instead of the client thinking:

“Why is Occu-Med taking so long?”

They see:

“Most delays are because applicants/providers have not submitted complete information.”

That protects Occu-Med.

22. Client Premium Package

This is the money part.
Occu-Med can create service tiers.

Basic RDQA Processing

Included in standard service:

- RDQA letter issued
- Applicant submits records by email/fax
- Manual follow-up as needed

Enhanced RDQA Support

Premium add-on:

- Applicant portal access
- Condition-specific instructions
- Provider packet generator
- Upload tracking
- Automated reminders
- Missing item checklist
- Client dashboard

Enterprise RDQA Management

For major clients:

- Custom-branded portal
- Client-specific rules
- Monthly analytics
- SLA reporting
- Priority review queue
- Dedicated RDQA case tracking

This gives Andy a revenue path.

23. Internal Training Tool for New EQA Analysts

This portal could double as a training system.
New analysts could search:
“Elevated ASCVD risk”
And the portal explains:

- What it usually means
- What documentation is usually requested
- What provider type is usually needed
- What common mistakes happen
- Sample applicant email
- Sample provider instruction sheet
- Sample missing-info follow-up

This reduces training time and improves consistency.

24. “RDQA Language Library”

Create a searchable internal library of standard wording.
Examples:

Category	Standard Language
Hypertension	3 readings, control, medication tolerance
Diabetes	A1c, complications, treatment, monitoring
Behavioral health	stability, symptoms, lethality, deployment stressors
Dental	DD2813 Category 1 or 2
ASCVD	cardiology evaluation, stress testing
Abnormal labs	provider comment on out-of-range values
Medications	side effects, special handling, impairing symptoms

This can support both:

- Human analysts
- AI-assisted generation

25. “Applicant Communication History” Timeline

Instead of digging through emails, EQA sees:

Date	Event
08/01	RDQA issued
08/02	Applicant opened portal
08/03	Provider packet downloaded
08/08	Applicant uploaded provider letter
08/09	EQA marked letter incomplete
08/09	Missing info message sent
08/11	Extension requested

This makes handoffs easier between analysts.

26. Auto-Generated “Missing Info” Follow-Up

Using Damian’s case as an example:
If hypertension information is incomplete, EQA selects missing items:

- End-organ damage
- 3 BP readings
- Medication side effects

- Monitoring plan

Portal generates:
Our subject matter experts reviewed the information submitted. The following items remain outstanding: documentation of blood pressure control and stability, at least 3 clinical blood pressure readings, medication list with side-effect statement, and provider comment regarding ongoing monitoring over the next 12 months.
This makes follow-up consistent and fast.

27. Provider Statement Template by Condition

This is huge.
The portal gives applicants a fillable template their provider can use.
Example:

Hypertension Provider Statement Template

I evaluated [Name] on [Date] regarding hypertension/elevated blood pressure.

1. Current diagnosis/status:
2. Blood pressure readings:
3. Evidence of end-organ damage:
4. Symptoms or complications:
5. Current medications:
6. Medication side effects:
7. Ongoing monitoring:
8. Restrictions/accommodations:
9. Ability to perform listed job duties:

Provider name/signature/date/credentials.
This could cut repeat submissions dramatically.

28. “Do Not Submit This” Examples

Applicants need examples of bad documentation.
The portal can show:

Usually Not Enough

“Patient is cleared to work.”
Why?
Because it does not address:

- Current condition status
- Job functions
- Restrictions/accommodations
- Symptoms
- Treatment
- Monitoring
- Required testing

Better

"The applicant's blood pressure is currently controlled on lisinopril 10 mg daily. Three clinical readings were 132/84, 128/82, and 130/80. The applicant denies dizziness, chest pain, syncope, or medication side effects. No work restrictions are recommended."

This helps applicants understand what "specific" means.

29. RDQA "Case Type" Classification

Every case can be tagged:

Case Type	Meaning
Simple documentation	Needs basic provider statement
Testing required	Needs labs, BP readings, stress test, CPAP report, etc.
Specialist required	Needs cardiology, neurology, behavioral health, dental, etc.
Waiver likely	Needs documentation for waiver authority
Client deadline risk	Deadline may expire before completion
High complexity	Multiple conditions, multiple providers, multiple tests

This helps managers assign workload more intelligently.

30. Management ROI Dashboard

Andy will likely care about numbers.

The portal should eventually show:

Metric	Why It Matters
Average emails per RDQA case	Shows EQA workload
Average days to complete RDQA	Shows turnaround
Percent of first submissions incomplete	Shows applicant/provider confusion
Most common missing items	Helps improve templates
Conditions causing longest delays	Helps forecast workload
Cases resolved without analyst intervention	Shows cost savings
Client-specific RDQA volume	Supports pricing discussions
Time saved per case	Justifies portal investment

The pitch should say:

"Even if the portal saves only 10–15 minutes per RDQA case, that becomes

significant at scale. If it prevents even one additional EQA hire, the portal pays for itself."

Best Version of the Product Concept

Name Ideas

- **Occu-Med RDQA Navigator**
- **Occu-Med Medical Follow-Up Portal**
- **RDQA Guidance Hub**
- **Occu-Med Applicant Resolution Center**
- **Occu-Med CaseComplete**
- **Occu-Med Follow-Up Navigator**

My favorite for a pitch:

Occu-Med RDQA Navigator

Because it sounds professional, supportive, and operational.

The Pitch Summary

You could tell Andy:

I think there is an opportunity to turn one of our most repetitive Exam QA pain points into a structured digital product. A large amount of RDQA time is spent explaining dense medical follow-up language to applicants, telling them what their provider needs to address, correcting incomplete provider letters, and sending repeated follow-ups.

I believe we could build an RDQA Navigator portal that gives applicants condition-specific guidance, generates provider instruction packets, tracks missing documentation, and gives clients visibility into where cases are delayed.

This could reduce repetitive analyst workload, improve turnaround time, standardize communication, reduce incomplete submissions, and create a premium service that Occu-Med could charge clients for as enhanced RDQA support.

The "Smart as Fuck" Core Feature Set

If you only pitch 10 things, pitch these:

Priority	Feature	Why It Matters
1	RDQA letter translator	Converts dense letters into plain English
2	Condition-specific checklists	Reduces confusion and incomplete records
3	Provider question packet generator	Makes provider responses more complete

4	Upload checklist	Prevents bad submissions
5	Missing-info detector	Reduces repeated SME/EQA review
6	Applicant next-step dashboard	Cuts “what do I do?” calls
7	Internal EQA smart reply generator	Saves analyst typing time
8	Deadline extension assistant	Standardizes extension requests
9	Client delay dashboard	Protects Occu-Med and adds client value
10	Premium client reporting package	Creates a way to charge more

What Makes This Different From a Normal Portal

A normal portal says:
“Upload your documents here.”
This portal says:
“Here is exactly what your provider must address, why your previous document may be incomplete, what your next step is, and what is still missing before Occu-Med can complete review.”
That is the difference.
That is what makes it worth money.

Outlook.pdf
PDF

Outlook(1).pdf
PDF

Outlook(2).pdf
PDF

Outlook(3).pdf
PDF

Outlook(4).pdf
PDF

Thought for a couple of seconds

Alex, **this Earl example is probably one of the strongest proof-of-concept cases** because it shows the exact breakdown:

1. **The RDQA letter contains dense official language.**
2. **The applicant/provider partially responds.**
3. **The SME determines the response is incomplete.**
4. **You then have to manually translate the request into plain English and explain what the provider still needs to address.**
5. **That extra email is basically the portal feature already being done manually.**

Your email to Earl directly explains that the clinic addressed only part of the diabetes inquiry, then gives him general provider-letter guidance and condition-specific diabetes instructions, including diagnosis, target organ damage, treatment, monitoring, diabetic foot exam, and nutrition counseling.

This Should Become a Signature Portal Feature

Feature Name: RDQA Condition Translator

The portal should take the official RDQA language:

Elevated hemoglobin A1c requires report from evaluation and statement from

personal physician addressing…
And convert it into applicant/provider-friendly language:

Diabetes / Elevated A1c Follow-Up

Your exam showed an elevated A1c result. Occu-Med needs your provider to review the result and explain whether it represents an underlying medical condition, such as diabetes or prediabetes.
Your provider should address:

Requirement	Plain-English Meaning
Whether the A1c result is diagnostic of a condition	Does this result mean you have diabetes, prediabetes, or another condition?
Target organ damage, complications, or symptoms	Are there kidney, nerve, eye, heart, circulation, or other complications?
Treatment instituted	Were medications, lifestyle changes, nutrition counseling, insulin, or other treatments started?
Ongoing monitoring over 1 year	Will you need follow-up visits, lab monitoring, medication management, or specialty care during deployment?
Initial diabetic evaluation	If newly diagnosed, were foot exam, eye exam, nutrition counseling, or other diabetes evaluations completed?

That is exactly what you manually wrote to Earl when you explained that the provider should review the A1c result and address diagnosis, complications, treatment, monitoring, diabetic foot exam, and nutrition counseling.

The Product Idea Gets Even Stronger

This should not just be a static help article.
It should work like this:

Step 1: Applicant Selects Their RDQA Condition

Example:
“Elevated A1c / Diabetes”

Step 2: Portal Shows What It Means

Your exam showed an elevated A1c result. Occu-Med needs additional information from your healthcare provider to determine the current status of the condition and whether it may affect your ability to safely perform the essential job functions of your role.

Step 3: Portal Generates a Provider Packet

The packet says:

Provider Response Requested: Elevated A1c / Diabetes

Please review the attached RDQA letter, A1c result, and essential job functions.
Please provide a statement addressing each item below:

1. Is the A1c result diagnostic of an underlying medical condition?
2. If yes, what is the diagnosis?
3. Is there any target organ damage, related complication, or impairing symptomatology?
4. Has treatment been started?
5. Please list all treatment, medication, lifestyle, or monitoring recommendations.
6. Will ongoing monitoring be required over the proposed deployment period?
7. If yes, how often will follow-up be needed?
8. Has a diabetic foot exam been completed?
9. Has nutrition counseling been completed?
10. Does the condition impact the applicant's ability to perform the listed job functions?
11. Are any restrictions or reasonable accommodations needed?

This converts the RDQA letter into an actionable form.

The Portal Should Also Have a "Provider Letter Completeness Check"

For Earl's case, the issue was that some diabetes information was submitted, but the original inquiry was **not fully covered**.
That should become a checklist:

Diabetes Documentation Completeness Check

Item	Complete?
Provider reviewed A1c result	☐
Diagnosis included	☐
Target organ damage addressed	☐
Related complications addressed	☐
Impairing symptoms addressed	☐
Treatment listed	☐
Ongoing monitoring addressed	☐
Follow-up frequency specified	☐
Diabetic foot exam included, if applicable	☐
Nutrition counseling included, if applicable	☐

Job functions addressed	☐
Restrictions/accommodations addressed	☐

Then the portal can say:
This submission may be incomplete. The provider did not address ongoing monitoring or whether the condition impacts the essential job functions.
That prevents the SME from reviewing incomplete documentation again.

Add This to the Andy Pitch

This example gives you a very clear business argument:

Current State

EQA analysts are manually:

- Translating RDQA language
- Explaining what the provider needs to address
- Repeating the same general guidance
- Creating condition-specific explanations
- Correcting incomplete provider responses
- Re-sending letters and job descriptions
- Telling applicants to point providers to page 2
- Clarifying that responses must relate to the actual job, not generic return-to-work status

Your Earl email is a perfect real example of this manual work. You had to tell him to give the provider the Occu-Med letter, point the provider to page 2, provide the email as additional guidance, and make sure the provider responded specifically to the diabetes/A1c issues.

Future State

The portal automatically generates:

- Applicant-friendly explanation
- Provider-facing checklist
- Job-function reminder
- Condition-specific documentation list
- Upload checklist
- Missing-item follow-up
- Readiness score

The Killer Sentence for Andy

Use this:
“The portal would take the type of manual clarification emails we are already writing for applicants and turn them into a standardized, condition-specific workflow that applicants and providers can follow before submitting documentation. This would reduce repeated EQA follow-up, improve first-pass documentation quality, and give clients a premium RDQA support option that can

be priced separately."
That is the pitch.
Not "we need a portal."
"We can productize the manual RDQA guidance work we already perform every day."

We also spend hours on the phone withg thewm.

Exactly — and that makes the portal pitch **way stronger**.
Because the real cost is not just emails. It is:
EQA analyst time spent on repeated live explanation calls that cannot be easily tracked, standardized, scaled, or billed separately.
That should be one of the main arguments to Andy.

Add This to the Pitch: Phone Time Is the Hidden Cost

Current Problem

RDQA applicants often call because they do not understand:

- What RDQA means
- Whether they are disqualified
- What page 2 of the letter is asking for
- Which provider they need to see
- What the provider letter should say
- Why "cleared to work" is not enough
- What documentation is missing
- Whether their deadline can be extended
- Whether they need restrictions or accommodations addressed
- Whether the provider needs to review the job description
- Whether their submitted records were enough

That turns EQA analysts into live RDQA translators.
And every call can easily become:

Call Type	Approx. Time
Basic "what does this mean?" call	10–15 minutes
Multi-condition RDQA explanation	20–45 minutes
Provider-letter coaching	15–30 minutes
Deadline/extension discussion	10–20 minutes
Follow-up after incomplete records	15–30 minutes
Upset/confused applicant call	30–60 minutes

Even a few calls per day per analyst becomes a huge hidden labor cost.

Portal Feature: "Before You Call" Guided Help

The portal should have a **Before You Call** section.
When an applicant clicks "I need help," it asks:

What do you need help with?

- I do not understand my RDQA letter
- I do not know what provider to see
- I need help explaining this to my provider
- My provider wrote a letter but I am not sure if it is enough
- I need more time
- I submitted records and want a status update
- I do not understand what is missing
- I have a question about restrictions/accommodations
- I am worried this means I am disqualified

Then the portal gives a guided answer first.
If they still need help, it creates a clean ticket for EQA instead of forcing a long call from scratch.

Portal Feature: Call Deflection Assistant

This is a very business-friendly term.

Call Deflection Assistant

The goal is not to block applicants from getting help.
The goal is to answer repeat questions before they require analyst time.
Example:
Applicant question:
"What do I ask my doctor for?"
Portal answer:
Bring your RDQA letter, job description, and this provider checklist. Ask your provider to respond directly to each item listed in your RDQA request. A general statement saying you are cleared to work may not be enough.
Then it shows the exact condition-specific checklist.
This could reduce unnecessary calls while still supporting applicants.

Portal Feature: "Call Prep Summary"

If the applicant still needs a call, the portal should make them answer a few questions first:

- Which condition are you calling about?
- Have you read the plain-English explanation?
- Have you scheduled a provider appointment?
- Have you uploaded any records?
- What part is confusing?
- Are you requesting a deadline extension?

Then EQA gets a summary before calling:

Applicant Call Summary

Field	Response

Applicant	Earl Fields
Condition	Elevated A1c / diabetes
Confusion	Provider letter incomplete
Appointment scheduled	Yes, PCP on 09/10
Documents uploaded	A1c lab only
Needs extension	Maybe
Main question	What should provider include?

Now the analyst does not have to spend 15 minutes just figuring out what the call is about.

Portal Feature: "Request a Callback" Instead of Random Calls

Instead of applicants calling repeatedly, portal could offer:

Request a Callback

Applicant selects:

- Reason for call
- Best time
- Condition
- Urgency
- Question

Then the system says:

Before your callback, please review your condition checklist and upload any documents you already have.

This creates better call control and reduces interruptions.

Portal Feature: Scripted Phone Guidance for EQA

For internal use, analysts should have guided call scripts.

Example:

Diabetes / Elevated A1c Call Script

1. Confirm applicant has the RDQA letter.
2. Direct them to page 2.
3. Explain that the provider must address each bullet point.
4. Explain that the provider should review job duties.
5. Explain that the provider must address:
    - Diagnosis
    - Complications
    - Treatment
    - Monitoring
    - Foot exam/nutrition counseling, if applicable
    - Restrictions/accommodations
6. Remind applicant that general clearance letters may not be sufficient.
7. Ask whether provider appointment is scheduled.

8. Ask whether deadline extension is needed.

This standardizes calls and helps new analysts.

Portal Feature: After-Call Auto Summary

This would be amazing internally.

After a call, the analyst selects checkboxes:

- Explained RDQA letter
- Directed applicant to page 2
- Explained provider letter requirements
- Discussed job description
- Discussed deadline
- Extension needed
- Applicant will follow up with PCP/cardiologist/dentist/etc.

The portal generates a note:

Spoke with applicant regarding elevated A1c RDQA request. Explained that provider must address diagnosis, complications/target organ damage, treatment, ongoing monitoring, and whether restrictions/accommodations are needed. Applicant was advised to provide RDQA letter, job duties, and guidance packet to PCP. Applicant plans to schedule appointment and will upload documentation once available.

That saves documentation time and creates a clean audit trail.

The Stronger Business Case

You can say:

"Right now, a large portion of EQA time is spent on phone calls explaining the same RDQA concepts over and over. These calls are necessary because the letters are dense, applicants are anxious, and providers often submit incomplete documentation. But this work is not scalable. A portal would allow us to shift the first layer of explanation into a guided self-service process, while still allowing applicants to request help when needed."

That sounds professional and strategic.

Best Andy Pitch Line

Use this:

"The biggest hidden cost is not only the number of RDQA emails we send. It is the amount of analyst time spent on live calls translating the same letter language into practical instructions. A portal could reduce those calls by giving applicants a condition-specific explanation, provider checklist, upload guidance, and next-step tracker before they ever need to call."

Even Better: Estimate the Cost

You can pitch it like this:

Example Cost Model

If one analyst spends:

- 2 hours per day on RDQA explanation calls
- 5 days per week
- 50 weeks per year

That is:

500 hours per year per analyst

If 3 analysts are doing that:

1,500 hours per year

That is almost a full-time workload just explaining RDQA requirements.

Even if the portal reduces calls by only 30%, that saves:

450 analyst hours per year

That is the kind of number Andy will care about.

Add This Feature Category

Applicant Self-Service + Call Reduction Tools

Feature	What It Saves
Plain-English RDQA explanation	Reduces “what does this mean?” calls
Condition-specific provider checklist	Reduces provider-letter coaching calls
Upload completeness check	Reduces repeat calls after incomplete documents
Deadline extension assistant	Reduces scattered deadline emails/calls
Callback request form	Reduces random interruptions
Call prep questionnaire	Shortens unavoidable calls
EQA call scripts	Standardizes analyst guidance
After-call summary generator	Reduces note-writing time
FAQ based on actual call reasons	Deflects repeat questions
Applicant status tracker	Reduces “any update?” calls

This is the missing part of the pitch.

YES — this one proves another **major portal category**:

Multi-Condition, Role-Specific RDQA Cases

This is not a simple “go get a doctor note” case. This is exactly why applicants panic and call for hours.

This one has:

Issue	What Makes It Complicated
Diabetes	Requires NFPA 1582 form, records, treadmill stress test report, testing, lab results
Elevated blood pressure	Requires 3 clinical BP readings over at least one week plus physician evaluation
Suspected sleep apnea	Requires treating physician documentation about current status and symptoms
Atrial fibrillation / Pradaxa	Requires physician explanation of the incident and ability to perform firefighting duties
Corrective lenses	Not disqualifying, but requires employee acknowledgement/ signature
Firefighter role	Requirements are tied to specific essential firefighting duties, not general work ability

That means the portal needs a feature specifically for **complex stacked RDQA cases**.

Feature: Multi-Condition RDQA Roadmap

Instead of dumping the applicant into a scary wall of text, the portal should break the letter into a roadmap.

Your RDQA Roadmap

You have **5 items** to address:

Item	What You Need	Provider / Action
Diabetes history	NFPA 1582 diabetes evaluation form, medical records, treadmill stress test report, recommended testing, labs	Treating physician
Elevated blood pressure	3 clinical BP readings over at least one week and medical evaluation	Physician / PCP

Suspected sleep apnea	Current status and symptom statement	Treating physician
Atrial fibrillation / Pradaxa	Incident details and ability to safely perform firefighter duties	Treating physician / cardiologist
Corrective lenses	Signature acknowledgment	Applicant

The portal should then show:
Start here: Schedule with your treating physician and bring the RDQA letter, NFPA 1582 form, job functions, and this checklist.

Feature: "One Appointment or Multiple Appointments?" Helper

Applicants constantly need to know whether one provider can handle everything. The portal should answer this.

Can One Provider Address This?

Condition	May Be Addressed By
Diabetes	Treating physician / PCP / endocrinologist
Blood pressure	PCP or treating physician
Sleep apnea	PCP, sleep medicine provider, pulmonologist, or treating physician with records
Atrial fibrillation / Pradaxa	Treating physician, often cardiologist if cardiac history is active or complex

Then the portal can say:
Some items may be addressed by the same provider if they have access to your records and are comfortable responding. However, cardiac-related items may require your treating cardiologist.
That saves long phone explanations.

Feature: Role-Specific Requirement Translation

This is important because the role is **firefighter**, not office work.
The portal should explain:
Your provider should not only comment on whether you are generally healthy or able to return to work. The provider must address whether the listed conditions could affect your ability to safely perform the essential firefighting job functions.
Then it should show the provider:

Please Consider Firefighting Duties Such As:

- Emergency response
- Physically demanding work

- Use of protective equipment
- Working under heat/stress
- Climbing, lifting, carrying, and rescue activity
- Operating in hazardous environments
- Maintaining alertness and rapid decision-making

That helps the provider answer correctly.

Feature: NFPA 1582 Guided Form Support

This example introduces a huge opportunity:

NFPA 1582 Form Assistant

For firefighter cases, the portal should:

- Explain what NFPA 1582 is in plain English
- Tell the applicant which attached form must be completed
- List what records must accompany it
- Remind them to include:
  - Medical records
  - Treadmill stress test report
  - Recommended testing
  - Lab results
  - Completed physician evaluation form

This is very different from a standard RDQA provider letter, so the portal should treat it as its own workflow.

Feature: Applicant "Document Bundle" Checklist

For this case, the portal should create a bundle checklist:

Required Uploads

Document	Required?	Status
Completed NFPA 1582 diabetes evaluation form	Yes	Missing
Diabetes-related medical records	Yes	Missing
Treadmill stress test report	Yes	Missing
Recommended testing results	Yes	Missing
Laboratory results	Yes	Missing
3 clinical BP readings	Yes	Missing

Physician blood pressure evaluation	Yes	Missing
Sleep apnea provider statement	Yes	Missing
Atrial fibrillation / Pradaxa provider statement	Yes	Missing
Corrective lenses acknowledgment	Yes	Missing

This is exactly the kind of thing applicants cannot track by themselves.

Feature: “Provider Packet by Condition”

For this case, the portal should generate separate packets:

1. **Diabetes / NFPA 1582 packet**
2. **Blood pressure packet**
3. **Sleep apnea packet**
4. **Atrial fibrillation / Pradaxa packet**
5. **Applicant signature acknowledgment**

Each packet should tell the provider:

- What condition is being addressed
- What job role is involved
- What form or testing is needed
- What exact questions need answers
- What supporting records should be included

This prevents one provider letter from vaguely addressing only one issue while ignoring the rest.

Feature: “Complexity Score”

This case should be marked high-complexity.

RDQA Complexity Score: High

Reasons:

- Multiple medical conditions
- Firefighter/NFPA standard involved
- Cardiac history involved
- Diabetes documentation involved
- Blood pressure readings required over time
- Possible sleep apnea documentation
- Applicant signature acknowledgment required

This helps EQA managers triage cases and assign workload appropriately.

Feature: “Call Risk Predictor”

This kind of case is almost guaranteed to generate calls.
The portal could flag:
High call-risk case: Applicant has multiple conditions and role-specific requirements. Send guided packet immediately and offer scheduled call only after applicant reviews checklist.
That is smart because it does not remove human help — it makes the human help more efficient.

The Pitch Point This Example Supports

Use this with Andy:
“Some RDQA letters include multiple conditions, specialty requirements, job-function analysis, attached forms, testing reports, and applicant acknowledgments. These cases almost always require long phone calls because the applicant cannot realistically understand what to do from the letter alone. The portal could break these complex cases into a step-by-step roadmap so the applicant knows which condition requires which provider, form, test, or signature before contacting EQA.”
That is a very strong operational argument.

Add This as a Premium Client Feature

For clients with public safety or physically demanding roles, Occu-Med could offer:

Role-Specific RDQA Workflow Packages

Examples:

Role Type	Custom Workflow
Firefighter	NFPA 1582 guidance, diabetes form, cardiac/BP/sleep apnea requirements
Police Officer	Pulmonary, cardiac, orthopedic, behavioral health, job-function analysis
DOT Driver	FMCSA-specific cardiovascular, medication, neurological, and driving safety language
Deployed Contractor	MOD 17/CENTCOM guidance, host-nation/deployment monitoring, waiver-readiness
Physical labor roles	Orthopedic restrictions, functional capacity, lifting/climbing/carrying

This gives Occu-Med a reason to charge more for advanced clients.

The New Portal Module List

Add these to the master plan:

1. **Multi-condition RDQA roadmap**
2. **One provider vs. specialist routing helper**
3. **Role-specific guidance engine**
4. **NFPA 1582 firefighter workflow**
5. **Document bundle checklist**
6. **Provider packet by condition**
7. **Complexity score**
8. **Call-risk predictor**
9. **Applicant signature acknowledgment tracker**
10. **Premium role-specific RDQA packages**

This is exactly the kind of feature set that makes the portal feel genuinely advanced, not generic.

Give me an insnaley strong and dertailed prompt for replit to build the app

Thought for a couple of seconds

Absolutely. Here is a **copy-and-paste Replit prompt** designed to force it to build the app as a serious operational product, not a cute generic portal.

Copy/Paste Prompt for Replit

```
Build a full-stack web application called “Occu-Med RDQA
Navigator.”

This application is for Occu-Med employees, applicants/
employees going through the RDQA process, and optionally
clients. The purpose is to reduce the amount of Exam QA analyst
time spent explaining RDQA letters by phone and email, while
helping applicants submit complete provider documentation the
first time.

The app must feel like a premium healthcare operations
platform, not a generic help center.

Use a polished, modern, luminous MacOS Tahoe / liquid-glass
design style:
- Dark navy / deep blue background
- Soft glowing orbs in the background
- Translucent glass cards
- Rounded corners
```

- Subtle gradients
- Clean professional typography
- Smooth hover states
- Luminous border glow on selected cards
- High-end SaaS dashboard feel
- Calm, medical, trustworthy, not flashy
- Must be responsive for desktop and mobile

Use React + TypeScript + Vite for the frontend.
Use Tailwind CSS for styling.
Use shadcn/ui style components if available.
Use lucide-react icons.
Use local mock data first so the app works immediately without needing external API keys.
Structure the app so it can later connect to Supabase/Postgres, authentication, file uploads, and AI assistance.

Do NOT build a simple landing page. Build an actual working prototype with multiple screens, dashboards, data models, and interactive workflows.

The main concept:
Occu-Med issues RDQA letters when applicants/employees need to submit additional medical documentation before a final recommendation can be made. These letters are often dense and hard for applicants to understand. Exam QA analysts spend hours on phone calls and emails explaining:
- What RDQA means
- What page 2 of the letter is asking for
- What provider the applicant needs to see
- What the provider letter needs to address
- Why “cleared to work” is not enough
- What documents are missing
- Whether the provider needs to review the essential job functions
- Whether restrictions or reasonable accommodations need to be addressed
- Whether additional testing is needed
- Whether a deadline extension is needed

This app should turn those repetitive explanations into a guided digital workflow.

The product should have these user types:

1. Applicant / Employee
2. Exam QA Analyst
3. Client / Employer
4. Admin

Create a role switcher in the prototype so I can preview each experience without building real authentication yet.

Core app sections:

1. Landing / Portal Home
2. Applicant RDQA Dashboard
3. RDQA Letter Translator
4. Condition-Specific Guidance Library
5. Provider Packet Generator
6. Upload / Completeness Checklist
7. Multi-Condition RDQA Roadmap
8. Request Callback / Before You Call Tool
9. Deadline Extension Assistant
10. Internal Exam QA Dashboard
11. Smart Reply Generator
12. Case Readiness Score
13. Client Dashboard
14. Admin Template Library

Use realistic mock cases and condition examples based on the RDQA process.

Important medical/legal positioning:
The app must clearly state that Occu-Med is not providing medical advice through the portal. The portal only helps applicants understand what documentation may be needed for Occu-Med's review process. The provider must make clinical assessments. Occu-Med reviews documentation against job/client/guideline requirements.

Use language like:
"This tool does not provide medical advice or determine qualification. It helps explain the documentation requested in your RDQA letter and organizes the information needed for review."

Applicant Dashboard Requirements:

Create an applicant dashboard showing:
- Applicant name
- Client/employer
- Job title
- RDQA status
- Deadline
- Days remaining
- Case complexity
- Next best action
- Required items
- Uploaded documents
- Missing items
- Provider packet download buttons
- Request callback button
- Deadline extension button

The dashboard should show one clear “Next Step” at the top, such as:
“Give the provider packet to your physician and ask them to respond to each item listed.”

Create progress cards:
- RDQA Letter Received
- Provider Packet Generated
- Provider Appointment Scheduled
- Documents Uploaded
- Completeness Review
- SME Review
- Final Recommendation

RDQA Letter Translator Requirements:

Create a screen where an EQA analyst or applicant can select a condition from a dropdown and see:
- Official RDQA language
- Plain-English explanation
- What this means
- What provider type may be needed
- What documents are needed
- What the provider should address
- Common mistakes
- Download provider checklist button

Include realistic condition templates for at least these conditions:

1. Elevated A1c / Diabetes
Official-style language:
“Elevated hemoglobin A1c requires report from evaluation and statement from personal physician addressing whether the results are diagnostic of an underlying medical condition, whether there is target organ damage, related complications, or impairing symptomatology, list of instituted treatment, and any ongoing monitoring indicated over the deployment term of 1 year. Newly diagnosed diabetic individuals should also have documentation of a complete initial diabetic evaluation, including eye exam, foot exam, nutrition counseling, etc.”

Plain-English version:
“Your A1c result was elevated. Occu-Med needs your provider to explain whether this means you have diabetes or another medical condition, whether there are any complications, what treatment was started, and whether you will need ongoing monitoring.”

Provider checklist:
- Does the A1c result indicate diabetes, prediabetes, or another diagnosis?
- Are there any complications such as kidney, nerve, eye, heart, circulation, or other target-organ issues?
- Are there any symptoms that could impair safe job performance?
- What treatment was started?
- What medication or lifestyle plan is being used?
- Is ongoing monitoring needed during the deployment or work period?
- If newly diagnosed, was a diabetic foot exam completed?
- Was nutrition counseling completed?
- Was an eye exam completed, if applicable?
- Does the condition impact the applicant’s ability to perform the essential job functions?
- Are restrictions or reasonable accommodations needed?

2. Elevated Blood Pressure / Hypertension
Official-style language:
“Elevated blood pressure warrants evaluation by physician and documentation of control. For reconsideration, employee should

provide at least 3 clinical blood pressure readings taken over a period of at least one week demonstrating control, and a report from a medical evaluation identifying the presence of any impairing symptomatology or target organ damage relevant to safe performance of essential job functions."

Plain-English:
"Your blood pressure readings were elevated. Occu-Med needs updated clinical readings and a provider statement explaining whether your blood pressure is controlled and whether there are symptoms or organ damage that could affect safe job performance."

Provider checklist:
- Provide at least 3 clinical blood pressure readings.
- Confirm whether blood pressure is controlled.
- Identify whether there is end-organ or target-organ damage.
- Address symptoms such as dizziness, chest pain, fainting, shortness of breath, etc.
- List medications.
- State whether medications cause impairing side effects.
- State whether ongoing monitoring is needed.
- Address restrictions/accommodations.
- Confirm whether essential job functions can be safely performed.

3. Hypertensive Urgency
Provider checklist:
- Presence of end-organ damage
- Control and stability
- At least 3 serial clinical BP measurements, for example ≤140/90 if applicable
- Complete medication list
- Medication side effects
- Hypertension-related symptoms or complications
- Recommended follow-up or monitoring over next 12 months
- Waiver support documentation if deployment-related

4. Behavioral Health / Depression / PTSD
Provider checklist:
- Current status and stability
- Date of diagnosis
- Baseline symptoms
- Residual symptoms

- Socially or occupationally impairing symptoms
- Any associated lethality/suicidality risk
- Treatment history
- Medication stability over past 90 days
- Response to treatment
- Ongoing care needed during deployment/work period
- Risk of worsening/decompensation under deployed or austere conditions
- Ability to perform essential job functions
- Restrictions/accommodations

5. Sleep Apnea
Provider checklist:
- Current status
- Treatment method
- CPAP use/compliance, if applicable
- Whether symptoms such as daytime sleepiness, impaired concentration, short-term memory issues, etc. exist
- Whether symptoms could inhibit safe performance of essential job duties
- Ongoing monitoring
- Restrictions/accommodations
- If CPAP compliance is required, include 30-day report showing at least 4 hours per night for at least 27 out of 30 days, if applicable

6. Dental / DD Form 2813
Plain-English:
"Your dental form indicates a dental status that may require re-evaluation. The dentist must complete an updated DD Form 2813 indicating Category 1 or Category 2 if no dental emergency is expected within the next 12 months."
Checklist:
- Updated DD Form 2813
- Category 1 or Category 2 marked, if appropriate
- Only one category box checked
- Dentist signature/date
- Dentist confirms whether emergency treatment is anticipated within 12 months
- If Category 3, explain treatment needed or complete treatment before reconsideration

7. ASCVD / Elevated Cardiovascular Risk
Checklist:

- Cardiological evaluation
- Stress echocardiography or graded exercise stress test with myocardial perfusion/SPECT if required
- Provider statement addressing findings
- Whether findings indicate treatment
- Whether cardiac condition interferes with ability to deploy/work
- Rationale if testing is not performed
- Restrictions/accommodations

8. Atrial Fibrillation / Pradaxa
Checklist:
- Specific circumstances surrounding incident
- Triggers
- Frequency of events
- Current treatment
- Medication use and side effects
- Ability to safely perform job functions
- Restrictions/accommodations

9. Abnormal Labs
Checklist:
- Provider comments on all out-of-range lab values
- Repeat testing if needed
- Diagnosis or explanation
- Treatment needed
- Ongoing monitoring
- Whether abnormalities affect job performance
- Restrictions/accommodations

10. Orthopedic Pain / Back / Ankle
Checklist:
- Functional limitations
- Risk of aggravation or exacerbation
- Ongoing treatment needed
- Type/frequency of care
- Impact on essential job tasks
- Medication side effects
- Restrictions/accommodations and expected duration

11. Pulmonary Function Concern
Checklist:
- Provider evaluation of pulmonary function testing
- Repeat PFT if reliability is questioned

- Whether impairment persists
- Whether impairment affects ability to safely perform role
- Restrictions/accommodations and duration
- Relevant medical history and testing

12. Nodule / Incidental Finding
Checklist:
- Diagnosis
- Whether further testing is needed
- Whether treatment is needed
- If no follow-up needed, provider should state that clearly
- Whether condition affects job functions
- Restrictions/accommodations

Provider Packet Generator Requirements:

Create an interactive packet generator where the user selects:
- Applicant name
- Job title
- Client/employer
- Condition(s)
- Deadline
- Provider type
- Whether job description is attached
- Whether labs/testing/forms are attached

Then generate a beautiful provider-facing packet preview with:
- Header
- Applicant information
- Role/job title
- “Purpose of this request”
- Required documents
- Provider questions
- Job function reminder
- Common mistakes to avoid
- Submission instructions
- Disclaimer

Include a button that says “Generate Packet” and then display the packet in a preview panel. No real PDF export is required yet, but structure it so PDF export can be added later.

The packet should include this provider explanation:
“Occu-Med is not asking you to make an employment

determination. Please document the individual’s current medical status, treatment, functional limitations, restrictions/accommodations, and whether the condition may affect the listed job functions. Occu-Med’s review team will evaluate the information against applicable client and job requirements.”

Upload / Completeness Checklist Requirements:

Create a mock upload screen. Since real file upload is not needed yet, let the user select mock uploaded documents:
- Provider Letter
- Lab Results
- BP Reading Log
- CPAP Compliance Report
- Dental Form DD2813
- Stress Test Report
- Job Description Reviewed Statement
- Medication List
- Specialist Evaluation

When a user selects documents, show a completeness checklist for the selected condition.

Example for Diabetes:
- Diagnosis addressed
- A1c reviewed
- Complications addressed
- Treatment listed
- Monitoring plan included
- Foot exam included if newly diagnosed
- Nutrition counseling included
- Job functions addressed
- Restrictions/accommodations addressed
- Provider name/date/signature included

Calculate a “Readiness Score” from 0 to 100.

Show statuses:
- Not ready for SME review
- Partially complete
- Likely ready but needs analyst check
- Ready for SME review

Use a circular progress indicator or progress bar.

Multi-Condition RDQA Roadmap Requirements:

Create a case example for a firefighter applicant with:
- Diabetes requiring NFPA 1582 evaluation form, medical records, treadmill stress test report, recommended testing, and lab results
- Elevated blood pressure requiring 3 clinical readings over at least one week and physician evaluation
- Suspected sleep apnea requiring physician documentation
- Atrial fibrillation / Pradaxa requiring treating physician statement
- Corrective lenses acknowledgment

Show this as a roadmap:
1. Diabetes / NFPA 1582
2. Blood Pressure
3. Sleep Apnea
4. Atrial Fibrillation
5. Corrective Lenses Signature

For each item show:
- Required provider
- Required document
- Status
- Difficulty
- Estimated delay risk
- Button to generate provider packet

Add “Complexity Score: High” and explain why:
- Multiple conditions
- Role-specific requirements
- NFPA firefighter standard involved
- Blood pressure readings over time
- Cardiac history
- Sleep apnea
- Applicant signature item

Before You Call / Request Callback Requirements:

Build a section that reduces phone calls.

When applicant clicks “I need help,” ask:
- I do not understand my RDQA letter

- I do not know what provider to see
- I need help explaining this to my provider
- My provider wrote a letter but I am not sure it is enough
- I need more time
- I submitted records and want a status update
- I do not understand what is missing
- I have a question about restrictions/accommodations
- I am worried this means I am disqualified

After selecting a reason, show:
- Suggested self-service answer
- Relevant checklist
- Button: “I still need a callback”

If callback is requested, collect:
- Best phone number
- Best time
- Condition
- Main question
- Whether provider appointment is scheduled
- Whether documents have been uploaded
- Whether deadline extension is needed

Then generate an internal call prep summary for EQA:
- Applicant
- Case
- Issue
- Condition
- Documents uploaded
- Main question
- Suggested call script

Deadline Extension Assistant Requirements:

Create a tool where applicants can request more time.

Fields:
- Current deadline
- Requested new deadline
- Reason for extension
- Provider appointment date
- Specialist appointment date
- Testing scheduled?
- Documents pending?

- Upload proof of appointment mock toggle

Generate an internal extension request summary:
“Applicant requests extension to [date] due to pending [provider/specialist] appointment on [date]. Documentation remains pending for [condition].”

Internal EQA Dashboard Requirements:

Create an analyst dashboard showing a table/list of RDQA cases.

Fields:
- Applicant name
- Client
- Job title
- Conditions
- Deadline
- Days remaining
- Status
- Readiness score
- Complexity
- Call risk
- Missing items
- Last contact
- Assigned analyst

Filters:
- High call risk
- Deadline approaching
- Missing provider letter
- Waiting on applicant
- Waiting on provider
- Ready for SME review
- Multi-condition cases
- Client
- Condition
- Complexity

Each case card should have:
- View roadmap
- Generate provider packet
- Generate smart reply
- Log call
- Request extension

- Mark ready for SME review

Smart Reply Generator Requirements:

Build an internal tool that generates draft emails based on selected scenario.

Inputs:
- Applicant name
- Condition
- Scenario
- Missing items
- Deadline
- Tone: warm, direct, formal
- Include job-function reminder?
- Include provider packet instructions?
- Include extension language?

Scenarios:
- Initial plain-English RDQA explanation
- Provider letter incomplete
- Missing documentation follow-up
- Dental form error
- CPAP compliance issue
- Hypertension/BP clarification
- Diabetes/A1c clarification
- Deadline extension follow-up
- Provider did not address job duties
- Provider did not address restrictions/accommodations

Generate realistic draft email text.

Example output for incomplete diabetes provider letter:
“Hello [Name], I’m following up regarding the information submitted for your elevated A1c/diabetes follow-up. After review, it appears that some aspects of the inquiry were not fully addressed. Please ask your provider to respond directly to the following items…”

Include copy button.

Call Script + After-Call Summary Requirements:

Create a “Log Call” feature.

For each condition, show a suggested call script:
- Confirm applicant has the RDQA letter
- Direct applicant to the second page
- Explain provider must answer each bullet point
- Explain job functions must be reviewed
- Explain that general “cleared” letters may not be enough
- Review condition-specific checklist
- Confirm provider appointment status
- Confirm deadline risk
- Ask if extension is needed

After call, allow analyst to check boxes:
- Explained RDQA process
- Directed applicant to page 2
- Explained provider letter requirements
- Explained job-function review requirement
- Discussed missing documents
- Discussed deadline
- Extension may be needed
- Applicant will follow up with provider

Generate after-call note:
“Spoke with applicant regarding [condition]. Explained that provider must address [items]. Applicant was advised to bring RDQA letter, job functions, and provider packet to appointment. Applicant plans to [next step].”

Client Dashboard Requirements:

Create a client-facing dashboard showing analytics:
- Total RDQA cases
- Cases pending applicant action
- Cases pending provider documentation
- Cases pending EQA review
- Cases ready for SME review
- Average days to resolution
- Top delay reasons
- Most common RDQA conditions
- Cases approaching deadline
- Percentage of incomplete first submissions
- Estimated analyst touchpoints avoided through portal

Use charts/cards.

Do not expose private medical details to clients in the mockup. Client view should show operational status and delay categories, not sensitive detailed medical information.

Client dashboard should support the business case that Occu-Med can charge more for:
- Enhanced RDQA support
- Client reporting dashboard
- Applicant guidance portal
- Premium analytics
- White-labeled client portal
- Faster turnaround support
- Reduced back-and-forth

Admin Template Library Requirements:

Create admin screens to manage:
- Condition templates
- Official language
- Plain-English explanation
- Provider questions
- Required documents
- Common mistakes
- Suggested provider types
- Call scripts
- Email templates
- Client-specific rules
- Role-specific workflows

Mock editing is enough. Make it look functional.

Sample mock cases to include:

1. Earl Fields
Client: Vectrus / LOGCAP
Role: Electrician, Lead in Kuwait
Conditions:
- Elevated A1c / Diabetes
- Category III dental status
- Body weight documentation
Status: Provider documentation incomplete
Readiness score: 52
Call risk: High

Deadline risk: Medium

2. Damian Farrar
Client: V2X
Role: Fire Chief
Conditions:
- Hypertensive urgency
- Elevated ASCVD risk
- Corrective lenses acknowledgment
Status: Waiver support documentation incomplete
Readiness score: 41
Call risk: High
Deadline risk: High

3. Firefighter Applicant
Client: Public Safety
Role: Firefighter
Conditions:
- Diabetes requiring NFPA 1582 form
- Elevated BP
- Suspected sleep apnea
- Atrial fibrillation / Pradaxa
- Corrective lenses acknowledgment
Status: Multi-condition roadmap needed
Readiness score: 28
Call risk: Very High
Complexity: Very High

4. Vicente Orozco
Role: Deputy Sheriff
Condition:
- Nodule / incidental finding
Status: Needs diagnosis and follow-up statement
Readiness score: 63
Call risk: Medium

5. Johnny Luevano
Role: Police Officer
Condition:
- Pulmonary function concern
Status: Needs provider evaluation and possible repeat PFT
Readiness score: 46
Call risk: Medium

6. Canet Espinoza
Role: HVAC Technician in Qatar
Conditions:
- PTSD history
- Abnormal laboratory results
Status: Needs updated evaluation and lab follow-up
Readiness score: 35
Call risk: High

The app navigation should include:
- Home
- Applicant Dashboard
- Letter Translator
- Provider Packet
- Upload Checklist
- Multi-Condition Roadmap
- Request Help
- EQA Dashboard
- Smart Replies
- Client Dashboard
- Admin Library

Home Page Requirements:

Create a strong home page with:
Title: Occu-Med RDQA Navigator
Subtitle: “Guided documentation support for faster, clearer, and more complete RDQA resolution.”
Cards:
- For Applicants: Understand what is needed and what to give your provider.
- For Exam QA: Reduce repetitive calls, emails, and incomplete submissions.
- For Clients: Gain visibility into RDQA delays and case movement.
- For Administrators: Standardize condition guidance and templates.

Add a business value section:
- Reduce repetitive phone calls
- Improve first-pass documentation quality
- Shorten RDQA turnaround time
- Standardize applicant guidance

- Reduce SME review of incomplete packets
- Improve client transparency
- Create premium chargeable service tiers

Add a mock ROI card:
“If each analyst spends 2 hours per day on RDQA explanation calls, that equals approximately 500 hours per analyst per year. Even a 30% reduction could save 150 hours per analyst annually.”

Design Requirements:

Make every screen visually polished.
Use:
- Glassmorphism
- Deep navy backgrounds
- Glowing blue/teal accents
- Subtle animated background orbs
- Translucent cards
- Rounded 2xl corners
- Soft shadows
- Sidebar navigation
- Dashboard cards
- Badges for status
- Progress bars
- Tables
- Interactive tabs
- Accordions for condition details
- Buttons with hover glow
- Clean icons

Avoid:
- Generic white bootstrap look
- Cluttered walls of text
- Unstyled forms
- Placeholder lorem ipsum
- Medical claims beyond documentation guidance
- Saying the app “clears” applicants

Implementation Requirements:

Build this as a working frontend prototype with mock data.
All screens should be clickable and navigable.
Use React Router if available; otherwise use state-based

navigation.
Create reusable components:
- GlassCard
- StatusBadge
- ProgressBar
- ConditionChecklist
- ProviderPacketPreview
- CaseCard
- DashboardMetric
- RoleSwitcher
- NavigationSidebar
- SmartReplyGenerator
- ReadinessScore
- TimelineStep
- ComplexityBadge

Use mock data arrays for:
- Conditions
- Cases
- Provider questions
- Required documents
- Email templates
- Call scripts
- Client analytics

The app should run immediately with:
npm install
npm run dev

Do not require real authentication, database, or API keys yet.
But structure the code so those can be added later.

Add comments in the code explaining where Supabase/auth/file upload/AI integration could be added later.

At the end, provide a short summary of:
- What was built
- How to run it
- What files contain the main data/templates
- What features are mocked
- What should be built next

Build the full prototype now.

Strong Add-On Prompt After It Builds

After Replit creates the first version, paste this as the **second prompt**:

```
Now improve the prototype so it feels much more premium,
operational, and executive-demo ready.

Focus on:
1. Making the UI look more like a polished MacOS Tahoe /
liquid-glass SaaS product.
2. Adding more realistic interactions, not just static pages.
3. Making the RDQA workflow feel genuinely useful to Occu-Med.

Specific improvements:

- Add animated glowing background orbs that move slowly.
- Add glass cards with blur, border highlights, and hover glow.
- Add a luminous selected state to sidebar navigation.
- Add better status badges for:
  - Waiting on applicant
  - Waiting on provider
  - Incomplete submission
  - Ready for SME review
  - Deadline risk
  - High call risk
- Add a “Next Best Action” component to every applicant case.
- Add a “Case Timeline” component with realistic RDQA events.
- Add a “Missing Items” drawer/panel.
- Add a “Generate Follow-Up Email” button on each missing item.
- Add a “Generate Provider Packet” button on each condition.
- Add tabs inside each case:
  - Overview
  - Conditions
  - Documents
  - Provider Packet
  - Communication
  - Timeline
  - Readiness
- Add mock uploaded document cards.
- Add a document completeness review panel.
- Add more realistic smart reply outputs.
- Add a call log panel with generated call summaries.
- Add an executive business value page for Andy.
```

```
Create a new page called “Executive Pitch” that summarizes:
- Current pain points
- Hidden phone-call cost
- EQA time savings
- Reduced incomplete submissions
- Faster RDQA turnaround
- Client-facing premium value
- Possible pricing tiers
- Implementation phases
- ROI estimate

Pricing tiers:
Basic: standard RDQA processing
Enhanced RDQA Support: applicant portal + provider packets +
upload checklist
Enterprise RDQA Management: client dashboard + analytics +
branded portal + priority support

Make sure the app still runs without external services.
```

Third Prompt: Make It Actually Pitchable to Andy

Use this after the UI is decent:

```
Add an executive demo mode designed for presenting this app to
Occu-Med leadership.

Create a guided demo flow with the following sections:

1. The Problem
Show that EQA analysts spend significant time explaining RDQA
letters by phone and email.
Include examples:
- Applicant does not understand what page 2 means
- Provider sends vague “cleared to work” letter
- Provider does not address job functions
- Records are submitted but incomplete
- Analyst must send repeated clarification emails
- Applicant calls repeatedly for status updates

2. The Solution
Show RDQA Navigator as a guided workflow:
- Letter translator
```

- Condition-specific checklist
- Provider packet generator
- Upload completeness check
- Readiness score
- Callback deflection
- Smart reply generator
- Client reporting dashboard

3. Before and After
Create a comparison:
Before:
- Dense RDQA letter
- Applicant confused
- Long phone call
- Incomplete provider letter
- Rework
- Delayed SME review

After:
- Plain-English explanation
- Provider checklist
- Structured upload
- Missing items identified early
- Applicant knows next step
- EQA saves time

4. ROI Calculator
Create a simple interactive ROI calculator with sliders:
- Number of EQA analysts
- Average RDQA calls per day
- Average minutes per call
- Average hourly labor cost
- Expected call reduction percentage

Output:
- Estimated weekly hours saved
- Estimated annual hours saved
- Estimated annual labor value saved

5. Client Revenue Opportunity
Show pricing tiers and what clients get:
- Enhanced applicant support
- Reduced delays
- Dashboard visibility

```
- Monthly RDQA analytics
- Premium reporting
- Branded portal

6. Implementation Roadmap
Phase 1:
- Internal template library
- Smart reply generator
- Condition guidance pages

Phase 2:
- Applicant portal
- Provider packets
- Upload checklist
- Callback requests

Phase 3:
- Client dashboard
- Analytics
- Client-specific branded portals
- AI-powered document completeness scanning

Make this page beautiful and persuasive.
```

Fourth Prompt: Add AI Features Later

Use this once the prototype is stable:

```
Now add a mocked AI layer to the RDQA Navigator prototype.

Do not connect to a real AI API yet. Simulate the AI outputs
with deterministic mock functions.

Add these AI-assisted features:

1. RDQA Language Extractor
User pastes official RDQA language.
The app identifies:
- Condition
- Provider type
- Required documents
- Required tests
- Required provider statements
- Restrictions/accommodations requirement
```

```
- Whether job functions must be addressed
- Whether waiver support may be needed

2. Plain-English Translator
The app converts official RDQA language into applicant-friendly
explanation.

3. Provider Checklist Generator
The app converts the RDQA language into direct provider
questions.

4. Missing Information Detector
User selects or pastes provider letter text.
The app compares it against the checklist and identifies
missing items.

5. Smart Reply Generator
The app generates applicant follow-up emails explaining missing
items.

6. Call Summary Generator
The analyst selects call outcomes and the app generates a clean
internal note.

7. Client Delay Summary Generator
The app summarizes why a case is delayed without exposing
sensitive medical details.

Clearly label these as “AI-assisted mockup” or “Simulated AI
output” in the prototype.

Add comments showing where OpenAI API, Azure OpenAI, or a
HIPAA-compliant/internal model endpoint could be connected
later.
```

My Honest Recommendation

Start with the **first prompt only**. Let Replit build the base app. Then use the second and third prompts to make it prettier and more pitch-ready.
Do **not** ask Replit to connect Supabase/auth/uploads/AI in the first attempt. That is where it will start breaking. Get the demo prototype first, then layer the backend later.

RDQA Trainin 2.pdf
PDF

Pages from Proposal to Provide Resource Sheets to RDQA Applicants and Employees.pdf
PDF

Another provider letter .pdf
PDF

OMED-Central Valley | PFT.pdf
PDF

RDQA Overview and Purpose.pdf
PDF

Guide to the Recommended Delayed Qualification Authorization.pdf
PDF

Improving RDQA Process Support.pdf
PDF

0 TDK RDQA 1Traditional

General	Unnamed: 1	Unnamed: 2	Unnamed: 3
Hernia	[TYPE] hernia identified during pre-placement medical examination requires that the applicant obtain evaluation by a general surgeon and release from the specialist addressing the applicant's ability to safely perform the [JOB CLASS] specific essential job duties (attached), with or without restrictions. The physician may indicate that surgical repair is indicated for unrestricted clearance to perform the [JOB CLASS] essential job functions (attached). Any statement from		Incomplete exam - Fitness and cardiovascular testing not performed due to elevated blood pressure. Individual must complete testing once cleared by personal physician.

	the surgeon endorsing release to safely perform essential job duties must address: a) whether the performance of essential job duties places the individual at a significantly increased risk of aggravation or exacerbation (e.g. incarceration) of the medical condition; and, b) whether restrictions or accommodations are necessary to permit safe job performance.		
Abnormal CXR	Possibly abnormal chest x-ray requires evaluation by personal physician and release to safely perform specific essential job duties (attached).		Inability to perform push-ups continuously for 30 seconds requires retesting to demonstrate ability to meet the push-up requirement.

Positive CTS (Other)	Positive Phalen's, Tinel's, Finkelstein's testing identified during pre-placement medical examination requires evaluation by a qualified physician (e.g. an orthopedist or neurologist). The physician must provide a statement addressing: a) the presence of any functional limitations (please identify if present); b) whether the performance of the [job class] essential job functions (attached) poses a significantly increased risk of injury, or aggravation or exacerbation of the condition (if any). If yes, please identify the potentially injurious job functions; and c) any work		Inability to perform sit-ups continuously for 30 seconds requires retesting to demonstrate ability to meet the sit-up requirement.

	restrictions or accommodations necessary to permit the safe performance of essential job functions. Please document the period of time that any restrictions or accommodations are expected to remain in effect.		

I think I like ur idea.. what it seems to be missing though is that we need to give them resources to address their coindirion. most of these peiple dont have the money to get extensive testing done or dont have insurancxe for a doctors visit. also please read all attached documents

Thought for 11s

Yes — **you are 100% right.** The app idea was missing the most human/practical layer:

The portal cannot only tell people what documentation they need. It also needs to help them figure out how to realistically get that documentation when they are uninsured, underinsured, broke, rural, or do not have a primary care provider.

I read through the attached documents enough to pull the important pattern: Occu-Med's training says applicants are financially responsible for follow-up, and Occu-Med generally cannot schedule or directly refer them because that can create conflict-of-interest concerns and because insurance/provider networks vary case by case. But the same training also says analysts should explain the provider type best suited for the follow-up and may point applicants toward competent resources they are familiar with.

That is the opening.

The portal should not be framed as:

"Occu-Med will find you a doctor."

It should be framed as:

"Occu-Med provides neutral resource guidance to help applicants understand possible low-cost, self-pay, sliding-scale, financial-assistance, and community-care options while preserving the applicant's right to choose

their own provider."
That solves the support problem without turning Occu-Med into a referral service.

The Missing Portal Module: Resource Navigator

This should become a major app section.

Occu-Med RDQA Resource Navigator

Purpose:
Help applicants identify realistic pathways to obtain the medical testing, provider evaluation, or documentation requested in their RDQA letter — especially when they do not have insurance, do not have a primary care provider, or cannot afford standard specialist testing.
This would directly address the real pain point from your proposal: applicants face significant challenges navigating the healthcare system, especially uninsured individuals who must obtain additional testing or evaluations at their own expense.

Add These Features to the Replit App

1. "I Don't Have a Doctor" Pathway

When an applicant opens a condition checklist, the portal should ask:
Do you currently have a provider who can address this condition?
Options:

Applicant Answer	Portal Response
Yes, I have a PCP	"Start with your PCP. Bring this packet."
Yes, I have a specialist	"This may be best addressed by your treating specialist."
No, I do not have a provider	"Here are low-cost ways to establish care."
I have insurance but do not know who accepts it	"Call your insurance/member services or use your plan's provider directory."
I am uninsured	"Here are self-pay, sliding-scale, FQHC, hospital charity-care, and community clinic options."
I cannot afford testing	"Here are financial assistance, payment-plan, and lower-cost testing options."

This is the missing user experience.

2. Resource Sheets Built Into the Portal

Your uploaded resource sheet is exactly the model. It includes a courtesy disclaimer, says Occu-Med does not endorse or partner with the providers, and

tells applicants to verify pricing and availability directly.
So the app should have a searchable resource database by:

- Region
- Zip code
- State
- Condition
- Test type
- Provider type
- Cost level
- Sliding scale availability
- Charity care availability
- Cash-pay pricing
- Transportation options
- Whether referral may be required

Example categories:

Resource Type	Examples
Community clinics	FQHCs, county clinics, sliding-scale clinics
Hospital financial assistance	Charity care, financial counseling, payment plans
Cash-pay testing	MDsave, Sesame, New Choice Health, Quest, LabCorp
Low-cost specialty care	Cardiology clinics, pulmonology clinics, sleep clinics
Free/low-cost clinics	Local free clinics, nonprofit clinics
University programs	Respiratory therapy, dental, optometry, health programs
Transportation resources	211, public transit, medical transport options
Condition-specific help	Diabetes clinics, dental schools, sleep centers, vision centers

Your Central Valley PFT sheet already has this structure: community health centers, hospital charity-care programs, diagnostic centers, urgent care, cash-pay discount websites, free/low-cost clinics, university/training programs, transportation resources, negotiation phrases, county programs, and questions to ask providers.

3. “Cheapest Practical Path” Tool

This is a very smart feature.
For each requirement, the app should suggest practical pathways, not just a

provider list.
Example:

Requirement: Elevated Blood Pressure

Lowest-cost path:

1. Ask a primary care clinic or community clinic about cash-pay BP evaluation.
2. Ask if they can document 3 clinical readings.
3. Ask if they offer sliding-scale pricing.
4. Ask if the provider can issue a statement addressing symptoms, medication side effects, target-organ damage, restrictions, and job functions.

Requirement: PFT

Lowest-cost path:

1. Call community health centers first.
2. Ask if they perform PFT or can refer to a low-cost respiratory testing site.
3. Compare cash-pay testing sites.
4. Ask whether the price includes interpretation/report.
5. Ask whether results can be released for employment documentation.

The uploaded PFT resource sheet even includes call questions such as cash-pay rate, additional fees, financial assistance, appointment timing, referral requirements, and result timing.
That should be in the portal.

4. Cost Barrier Intake

When someone clicks “I need help finding care,” the app should ask:

What is making this difficult?

- I do not have insurance
- I have insurance but cannot afford the copay
- I do not have a primary care provider
- I cannot get an appointment before the deadline
- I need a specialist and cannot afford it
- The provider requires a referral
- The provider does not understand the letter
- I live in a rural area
- I do not have transportation
- I do not know what test to ask for

Then the portal gives targeted help.
This is way better than a static PDF resource sheet.

5. Resource Recommendation Without Referral Risk

The language matters.
The app should say:

These resources are provided as a courtesy only. Occu-Med does not endorse, recommend, contract with, or partner with these providers for your follow-up care. You may choose any qualified provider. Please verify current pricing, availability, referral requirements, and insurance acceptance directly with the facility.
This matches the disclaimer style in your PFT sheet.
That allows Occu-Med to support applicants without “owning” the appointment.

6. “Questions to Ask When Calling” Scripts

This needs to be built directly into every resource page.
Examples:

For Any Provider

- What is your cash-pay rate?
- Do you offer sliding-scale pricing?
- Do you offer financial assistance?
- Do I need a referral?
- Can the provider issue a written statement for employment/medical review purposes?
- Can the provider review my job description?
- Can I bring a letter that lists the exact questions the provider needs to answer?
- How soon can I get an appointment?
- How long will results take?
- Are there separate facility, interpretation, or report fees?

For PFTs and stress tests, this matters a lot because the hidden cost may be the interpretation/report, not just the test.
Your resource sheet already includes negotiation phrases like “I’m uninsured and this is for employment — do you have any special rates?” and “I’m paying cash and can settle today — what’s your best rate?”
That should be an in-app “Call Script” button.

7. “Can My PCP Do This?” Helper

Applicants constantly ask whether they need a specialist.
The portal should help sort this:

Requirement	Start Here	May Need Specialist If
Elevated BP	PCP / community clinic	Severe readings, cardiac symptoms, end-organ damage
Diabetes / A1c	PCP / endocrinologist	Complications, insulin complexity, poor control

Abnormal EKG	PCP or cardiology	Functional cardiac testing required
ASCVD risk	Cardiology	Stress echo/SPECT required
Sleep apnea	PCP / sleep medicine	CPAP compliance or sleep study needed
PFT concern	PCP / pulmonology / occupational medicine	Abnormal results persist
Dental status	Dentist / dental school / community dental clinic	Treatment needed
Behavioral health	Treating mental health provider	Recent instability, deployment stress concerns
Nodule/incidental finding	PCP	Imaging/specialist follow-up required

This fits the training because analysts are expected to explain the type of provider best suited to provide the information, while still preserving applicant choice.

8. “Affordable Care Pathways” Library

The portal should not just list clinics. It should teach people the different pathways.

Pathway A: Existing Treating Provider

Best for chronic conditions because they already know the history.

Pathway B: Federally Qualified Health Center / Community Clinic

Good for uninsured or low-income applicants.

Pathway C: County Health Department

Useful for vaccines, TB, some screenings, and public health resources.

Pathway D: Hospital Financial Assistance / Charity Care

Useful for expensive imaging, cardiac testing, specialty testing.

Pathway E: Cash-Pay Discount Sites

Useful for labs, imaging, some procedures.

Pathway F: Retail / Urgent Care

Useful for simple tests, basic evaluations, BP checks, vaccinations, EKGs.

Pathway G: Dental Schools / Teaching Clinics

Useful for dental forms, dental evaluation, lower-cost dental care.

Pathway H: University / Training Programs

Useful for PFTs, vision, hearing, dental, respiratory therapy, and lower-cost supervised services.

Your PFT sheet includes university/training programs and mobile health units as resource options, which is exactly the kind of “practical pathway” that belongs in

the portal.

9. Resource Matching by Condition

The app should map conditions to resource types.

Example: Diabetes / Elevated A1c

Show:

- Community health centers
- Low-cost primary care clinics
- Diabetes education programs
- Nutrition counseling resources
- Lab options for A1c
- Eye exam resources
- Foot exam/podiatry options
- Prescription discount resources
- Hospital charity-care programs

Example: Elevated BP

Show:

- Community clinics
- Low-cost PCP options
- Retail clinics
- Blood pressure reading locations
- Medication discount resources
- Labs if required
- Cardiology options if severe

Example: Dental Category III

Show:

- Community dental clinics
- Dental schools
- Low-cost dental programs
- Cash-pay dental exams
- DD2813 form guidance
- “Ask if they can complete military/employment dental forms”

Example: PFT

Show:

- Community clinics
- Pulmonology offices
- respiratory testing labs
- hospital financial assistance
- training programs
- cash-pay PFT options

This makes the portal genuinely useful.

10. “Deadline Reality Check”

If the person has no doctor and needs a stress test in 10 days, the portal should flag:
This requirement may be difficult to complete before your current deadline.
Consider requesting an extension after scheduling your appointment.
Then it helps them generate the extension request.
This is huge for reducing panic calls.

Updated Replit Prompt Add-On

Paste this into Replit **after the first app prompt**:

```
Update the Occu-Med RDQA Navigator prototype to add a major new
module called “Resource Navigator.”

This is extremely important.

The app should not only explain what documentation applicants
need. It must also help applicants understand realistic
pathways for obtaining the required medical evaluation,
testing, or documentation when they are uninsured,
underinsured, do not have a primary care provider, cannot
afford a specialist, live in a rural area, or do not know where
to start.

This must be framed carefully:
- Occu-Med is not providing medical advice.
- Occu-Med is not endorsing, recommending, referring to, or
partnering with specific providers.
- Occu-Med is not scheduling appointments.
- Applicants may choose any qualified provider.
- Resources are provided as a courtesy to help applicants
identify possible options.
- Applicants must verify current pricing, availability,
insurance acceptance, referral requirements, and services
directly with each facility.

Add a new main navigation item:
“Resource Navigator”

Resource Navigator must include:

1. Access Barrier Intake
Ask the applicant:
```

- Do you currently have a healthcare provider?
- Do you have insurance?
- Can you afford a doctor visit?
- Do you need help finding low-cost testing?
- Do you live in a rural area?
- Do you need transportation help?
- Do you know what type of provider you need?
- Are you close to your deadline?
- Do you need a specialist?
- Did a provider tell you they cannot complete the request?

Based on answers, show a “Recommended Resource Pathway.”

2. Resource Pathways
Create cards for:
- Existing Treating Provider
- Community Health Center / FQHC
- County Health Department
- Hospital Financial Assistance / Charity Care
- Cash-Pay Discount Testing
- Retail Clinic / Urgent Care
- Dental School / Community Dental Clinic
- University / Training Program
- Mobile Health Unit / Health Fair
- Transportation Assistance
- Prescription / Medication Discount Programs

Each card should explain:
- Best for
- Typical use cases
- Questions to ask
- Cost considerations
- Important warnings
- Whether a referral may be needed

3. Condition-to-Resource Matching
For each RDQA condition, show recommended resource options.

Examples:

Diabetes / Elevated A1c:
- PCP or community clinic
- FQHC
- Low-cost lab options

- Diabetes education / nutrition counseling
- Eye exam resources
- Foot exam / podiatry resources
- Medication discount resources

Elevated Blood Pressure:
- PCP or community clinic
- Retail clinic for BP readings
- FQHC
- Medication discount resources
- Cardiology if severe or complicated

PFT / Pulmonary Function Concern:
- Community clinic
- Pulmonology office
- Respiratory testing lab
- Hospital financial assistance
- Respiratory therapy training programs
- Cash-pay testing websites

Dental / DD2813:
- Community dental clinic
- Dental school
- Low-cost dental exam
- Cash-pay dental offices
- DD2813 form guidance

EKG / Cardiac / ASCVD:
- Community clinic for basic EKG
- Cardiology clinic
- Hospital financial assistance
- Cash-pay diagnostic tools
- Stress test resources

Behavioral Health:
- Existing treating provider
- Community mental health clinic
- County behavioral health
- Sliding scale therapy/psychiatry
- Telehealth if appropriate

Sleep Apnea:
- Existing sleep doctor
- PCP

- Sleep clinic
- CPAP supplier report resources
- Low-cost sleep study options if needed

4. Region-Based Resource Directory
Create mock resource data for regions:
- Central Valley
- Bay Area
- Southland / Los Angeles
- Central Coast
- Sierra / Gold Country
- Rural / Remote Area

Each region should include sample resource categories:
- Community clinics
- Hospital financial assistance
- Cash-pay discount websites
- Free/low-cost clinics
- Urgent care
- Diagnostic centers
- Transportation options
- County resources
- University/training programs

Use realistic mock resource cards with:
- Name
- Resource type
- Region
- Phone
- Website
- Typical cost range
- Sliding scale available
- Financial assistance available
- Services
- Notes
- “Verify before scheduling” reminder

5. Questions to Ask When Calling
Create a reusable call script generator.

Questions:
- What is your cash-pay rate?
- Do you offer sliding-scale pricing?
- Do you offer financial assistance or charity care?

- Do I need a referral?
- Is the provider able to issue a written statement for employment medical review purposes?
- Can the provider review a job description?
- Can the provider respond to specific questions listed in a letter?
- Are there separate facility, interpretation, or report fees?
- How soon can I get an appointment?
- How long will results take?
- Can I receive a copy of the results?
- Do you accept walk-ins?
- What should I bring?

Add condition-specific call scripts:
- PFT
- EKG
- Stress test
- Dental/DD2813
- Diabetes/A1c
- Blood pressure readings
- Behavioral health
- Sleep apnea

6. Cost-Saving Strategy Panel
Add tips:
- Ask for cash-pay pricing.
- Ask about same-day payment discount.
- Ask about sliding-scale programs.
- Ask hospital financial counseling before scheduling.
- Ask whether bundled testing is cheaper.
- Ask whether there are separate interpretation/facility fees.
- Compare nearby cities.
- Confirm whether written reports are included.
- Get quote in writing.
- Bring the RDQA letter and provider packet.

7. Deadline Risk Helper
If the applicant says they do not have a provider, cannot afford testing, or needs a specialist close to the deadline, show:
“This may be difficult to complete before your current deadline. Consider scheduling the soonest available appointment and requesting a deadline extension.”

Add button:
“Generate Extension Request”

8. Neutrality / Compliance Disclaimer
Display prominently:
“These resources are provided as a courtesy only. Occu-Med does not endorse, recommend, contract with, or partner with these providers for your RDQA follow-up. You may choose any qualified provider. Please verify pricing, availability, referral requirements, insurance acceptance, and services directly with each facility. This tool does not provide medical advice or determine qualification.”

9. Internal Admin Resource Library
Allow admin to add/edit resource cards:
- Region
- Service type
- Provider/resource name
- Phone
- Website
- Cost range
- Sliding scale
- Financial assistance
- Notes
- Last verified date
- Verification status
- Tags

Add verification statuses:
- Verified recently
- Needs verification
- Applicant-reported
- Public information only
- Do not display externally

10. Client Value / Executive Pitch Update
Update the Executive Pitch page to include:
- Applicants are often uninsured or underinsured.
- Applicants may lack a PCP.
- Applicants often cannot afford specialist visits or testing.
- Current process can feel like “you must solve this alone.”
- Resource Navigator shifts the experience from enforcement to guided support.

```
- This can reduce deadline extensions, improve applicant
satisfaction, reduce phone calls, and improve client workforce
planning.
- Resource guidance can become a premium client service without
becoming a referral service.

Make the design match the existing liquid glass / MacOS Tahoe
style.
```

The Pitch Becomes Much Stronger Now

The real product is no longer just:
“Here is what your provider needs to write.”
It becomes:
“Here is what you need, why you need it, what kind of provider can help, where you may be able to find low-cost options, what to ask when you call, how to avoid incomplete documentation, and how to request more time if access barriers make the deadline unrealistic.”
That is a much more humane, practical, and sellable product.
And it fits Occu-Med’s RDQA purpose because RDQA was created partly because clients do not have the training to guide applicants and to protect clients from unnecessary private health information exposure.

The Andy Pitch Sentence

Use this:
“The portal should not only translate the RDQA letter. It should also reduce the real access barriers that cause delays — no insurance, no PCP, unaffordable testing, rural locations, and confusion about where to start. We can provide neutral, non-referral resource guidance that preserves applicant choice while helping them move faster through the individualized assessment process.”
That is the missing piece.

Yes, the goal is threefold. Decrease the amount of call volume and stuff that ExamQR gets for routine same questions over and over again when an applicant has this. As someone who's been through a process, for example, I want to open a portal or a website or even Google something to see what is the best way to achieve this. There's nothing about this process online because it's very unique to Occumed. So we need to make something accessible. But here's the thing, we're also offering additional support, like, you know, submit your um additional information so that we can see if you have sufficient information to clear the condition, review it. It could be through the portal and then we could just provide them feedback as subject, as their case manager. Um, and I don't know, I'm thinking of ways to like sort of like pitch this to Andy as well as like a addition service so that they can charge for it, like pitch it to employers of like, you know, like, yeah, like we'll give them something so that they don't start from zero, they

don't start from scratch and all that stuff.

Thought for a second
Yes — **that is the actual product.** Not just an RDQA portal. Not just resource sheets. Not just a case tracker.
What you are describing is a **three-part support system**:

Occu-Med RDQA Navigator

A self-service support, document review, and client-value platform for RDQA cases

The goal is:

1. **Reduce ExamQA call volume**
2. **Give applicants/employees a real place to start**
3. **Create a premium service Occu-Med can charge employers for**

And honestly, that framing is much stronger than “we should make a website.”

The Core Problem

Right now, RDQA is unique to Occu-Med. There is no normal online search result that explains:

- What RDQA means
- Why the applicant received it
- Whether they are disqualified
- What kind of provider they need
- What their provider needs to write
- Why “cleared to work” is not enough
- What to do if they have no insurance
- What to do if they cannot afford testing
- What to do if they do not have a PCP
- How to know if the documentation is complete

So applicants naturally call ExamQA because **Occu-Med is the only source of explanation.**
That is the key sentence.
Because RDQA is an Occu-Med-specific process, applicants cannot meaningfully self-educate online. Without a self-service resource, ExamQA becomes the default help desk for every routine question.
That is the problem Andy will understand.

Why the Current Process Creates Calls

Occu-Med’s own RDQA training explains that applicants are financially responsible for additional follow-up testing/evaluation, and that Occu-Med generally does not schedule appointments or provide referrals because applicants may need to use their own provider, insurance network, or preferred care pathway.
That creates a practical gap:

Occu-Med tells the applicant:

"You need to obtain additional medical information."

But the applicant hears:

"I have to figure this out alone, pay for it, find the right doctor, explain this weird letter, and hope I submit the right thing before the deadline."

That gap is what creates phone calls, frustration, extensions, incomplete submissions, and repeated emails.

The Threefold Goal

1. Reduce Routine ExamQA Calls

The portal answers the same questions applicants repeatedly call about:

Applicant Question	Portal Feature
What is RDQA?	Plain-English RDQA explainer
Am I disqualified?	"RDQA does not mean disqualified" page
What does page 2 mean?	RDQA letter translator
What do I ask my doctor for?	Provider packet generator
What provider do I need?	Provider-type guidance
I have no doctor. What do I do?	Resource Navigator
I cannot afford testing. What now?	Low-cost care pathways
Is my provider letter enough?	Upload/document completeness review
What is missing?	Missing-item feedback
I need more time.	Deadline extension assistant
What is my status?	Case tracker

The goal is not to eliminate human support. The goal is to stop every applicant from needing a 30-minute phone call just to understand step one.

2. Give Applicants a Real Starting Point

This is the human part.

You said it perfectly: if you were going through this, you would Google it. But there is nothing to Google.

So Occu-Med can create the official place applicants go.

Applicant-facing portal should answer:

- What is this process?
- Why did this happen?
- What do I need to do next?
- What should I give my provider?
- What kind of provider can help?
- What if I have no insurance?

- What if I cannot afford the test?
- What if I need an extension?
- How do I avoid submitting something incomplete?

This shifts the applicant experience from:
"You are delayed. Figure it out."
to:
"You are delayed, but here is a guided path to help you move forward."
That is a major improvement.

3. Create a Chargeable Client Service

This is where the pitch becomes business-oriented.
Occu-Med can offer employers an enhanced RDQA support package.
Not just:
"We issue RDQA letters."
But:
"We provide a structured applicant support pathway that helps employees understand the follow-up process, identify practical resource options, submit documentation, receive completeness feedback, and reduce delays."
That can absolutely be positioned as a premium service.

The Service Model

I would pitch this as a new service tier:

Enhanced RDQA Support Program

Included Features

Feature	Value to Applicant	Value to Client	Value to Occu-Med
RDQA explainer portal	Less confusion	Fewer complaints	Fewer calls
Condition-specific guidance	Clearer next steps	Faster follow-up	Less repeated explanation
Provider packet generator	Better doctor letters	Fewer incomplete submissions	Less rework
Resource Navigator	Helps uninsured/ underinsured applicants	Fewer deadline extensions	Better applicant experience

Upload review	Applicant knows what may be missing	Faster case movement	Fewer SME reviews of incomplete records
Case status tracker	Less anxiety	More transparency	Fewer “any update?” calls
Extension assistant	Cleaner requests	Better workforce planning	Less messy email traffic
Client dashboard	Operational visibility	Premium reporting	Chargeable service

The Strongest Version of the Pitch to Andy

You can say:
“Right now, ExamQA is absorbing a large amount of routine support work because RDQA is an Occu-Med-specific process that applicants cannot research on their own. When applicants receive an RDQA letter, they often do not know what it means, what provider to see, what the provider needs to write, whether their documentation is sufficient, or how to proceed if they do not have insurance or cannot afford testing. As a result, analysts spend significant time on repetitive calls and emails explaining the same process case by case.”
Then:
“I think we can turn that pain point into a structured service. An RDQA Navigator portal could provide plain-English explanations, condition-specific provider packets, low-cost resource pathways, upload/completeness review, deadline-extension support, and client-facing status visibility. This would reduce routine ExamQA call volume, help applicants avoid starting from zero, and give Occu-Med a premium RDQA support product that can be offered to clients as an enhanced service.”
That is very strong.

The Missing Add-On: “Pre-Review Before SME Review”

This is the part you just added, and it is important.
You are saying:
Applicants could submit additional information through the portal, and Occu-Med could provide feedback on whether the documentation appears complete enough to move forward.
This should be called:

Documentation Completeness Review

Not “clearance review.”
Not “medical approval.”
Not “condition clearance.”
Use careful wording:
“Occu-Med can review submitted documentation for completeness against the

RDQA request before routing it for final medical review."
That is safer and more operationally accurate.

What this feature does

Applicant uploads:

- Provider letter
- Lab result
- BP log
- CPAP report
- DD2813
- Stress test
- Specialist evaluation
- Medication list
- Job-function statement

ExamQA or the portal checks:

Completeness Question	Example
Did provider address each bullet?	Yes/No
Did provider mention job functions?	Missing
Did provider address restrictions/ accommodations?	Missing
Did provider include test results?	Complete
Did provider include diagnosis/ current status?	Complete
Is provider name/date/signature present?	Complete
Is additional testing still needed?	Maybe

Then applicant gets feedback:
"Your submission appears to be missing a statement about whether restrictions or accommodations are needed. Please ask your provider to address this before resubmitting."
This can prevent the situation where SMEs review something and send it back again.

Why This Is Valuable to Clients

Clients do not just want Occu-Med's internal turnaround clock paused. Your uploaded proposal already makes that point: even if RDQA pauses Occu-Med's formal turnaround time, the employer is still waiting in the real world while the applicant tries to obtain follow-up information, often requesting extensions because of access barriers.
So the client value is:

- Fewer stalled applicants
- Fewer deadline extensions

- Fewer frustrated candidates
- Better workforce planning
- Less confusion about why cases are delayed
- Better documentation before final review
- More defensible individualized assessment process

This is not just applicant support. It is operational risk reduction for the employer.

How Occu-Med Can Charge for It

Option 1: Per-RDQA Enhanced Support Fee

Charge per applicant who enters the enhanced support workflow.
Example service:
Enhanced RDQA Support: includes portal access, provider packet, resource navigator, upload completeness review, and case-status guidance.

Option 2: Client Subscription

For large clients:
Monthly RDQA Support Dashboard + applicant portal access + reporting.

Option 3: Premium Client Tier

Basic clients get normal RDQA letters.
Premium clients get:

- Applicant portal
- Resource Navigator
- Document upload
- Completeness pre-review
- Client dashboard
- Analytics
- Branded support page

Option 4: White-Labeled Employer Portal

For major clients:
"[Client Name] Medical Follow-Up Support Center powered by Occu-Med."
This makes Occu-Med look more sophisticated and gives the employer something tangible.

The Portal Should Have 4 Main Applicant Tools

1. Understand My RDQA

Explains:

- What RDQA means
- Why they received it
- Why this does not automatically mean disqualification
- What page 2 means
- What the deadline means
- What happens next

The guide you uploaded already uses this framing: RDQA gives applicants the

chance to provide additional medical information instead of being outright disqualified, while the final employment decision remains with the employer.

2. Prepare for My Provider Visit

Generates:

- Provider packet
- Condition checklist
- Job-function reminder
- Questions the provider must answer
- Common mistakes to avoid

3. Find Practical Resources

Helps with:

- No PCP
- No insurance
- Low-cost clinics
- Sliding-scale options
- Hospital financial assistance
- Cash-pay testing
- Dental schools
- Community clinics
- Transportation resources
- Questions to ask when calling

The PFT resource sheet you uploaded already shows this model by listing affordable testing options, community health centers, hospital charity-care programs, cash-pay resources, transportation options, and questions to ask providers.

4. Submit and Check My Documents

Allows:

- Uploading documents
- Selecting which condition they address
- Seeing what is still missing
- Getting case-manager feedback
- Requesting an extension if needed

Internal ExamQA Tools

For ExamQA, this portal should reduce work but also make remaining work faster.

ExamQA Dashboard

Shows:

- Cases pending applicant action
- Cases with uploaded documents awaiting completeness review
- Cases with missing information
- High call-risk cases
- Deadline-risk cases

- Cases ready for SME review

Smart Feedback Templates

When a document is incomplete, analyst selects missing items and the portal generates feedback.
Example:
"The provider statement was received; however, it does not appear to address whether the condition requires restrictions or accommodations, and it does not confirm that the provider reviewed the essential job functions. Please ask your provider to amend the statement to address those items."
This replaces long manual emails.

Client-Facing Dashboard

Clients should not see unnecessary medical details.
They should see operational status:

Metric	Example
RDQA cases active	42
Pending applicant action	18
Pending provider documentation	11
Pending Occu-Med review	6
Ready for SME review	4
Deadline extensions requested	9
Average days in RDQA	8.7
Most common delay reason	Incomplete provider letter

This protects PHI while giving employers visibility.

Best Product Name Options

My favorites:

1. **Occu-Med RDQA Navigator**
2. **Occu-Med Follow-Up Navigator**
3. **Occu-Med CaseComplete**
4. **Occu-Med RDQA Support Center**
5. **Occu-Med Medical Follow-Up Portal**

For Andy, I would use:

Occu-Med RDQA Navigator

Enhanced Applicant Support & Documentation Guidance Platform

It sounds polished, serious, and sellable.

The Pitch in One Clean Paragraph

Use this:
RDQA is a valuable process because it gives applicants and employees the

opportunity to provide additional medical information before a final recommendation is made. However, because the process is unique to Occu-Med and not something applicants can easily research online, ExamQA becomes the default help desk for routine questions. Applicants often need help understanding the letter, identifying the right provider, explaining the request to their provider, finding affordable care, submitting documentation, and knowing whether their response is complete. I believe Occu-Med can turn this pain point into an enhanced service by creating an RDQA Navigator portal that provides plain-English guidance, provider packets, resource pathways, document upload/completeness review, status tracking, and client reporting. This would reduce routine call volume, improve applicant support, reduce incomplete submissions, and create a premium service Occu-Med can charge employers for.

The Executive Pitch Structure

When pitching Andy, frame it as:

1. Current Pain Point

ExamQA spends hours on repetitive calls/emails because applicants do not understand the RDQA process and have no public resource to consult.

2. Root Cause

RDQA is unique, dense, and often requires provider documentation/testing that applicants may not know how to obtain.

3. Applicant Barrier

Many applicants are uninsured, underinsured, lack a PCP, cannot afford specialist care, or do not know where to start.

4. Business Impact

This causes call volume, incomplete submissions, extensions, delayed final recommendations, client frustration, and inefficient analyst workload.

5. Proposed Solution

Build an RDQA Navigator portal with explanation, provider packets, resource guidance, upload completeness review, and status tracking.

6. Revenue Opportunity

Offer it as an enhanced RDQA support tier for clients.

7. Expected Benefit

Lower call volume, faster documentation completion, improved applicant experience, better client visibility, and a stronger premium service offering.

Add This to the Replit Prompt

Here is the **exact add-on prompt** to update the app concept:

```
Expand the Occu-Med RDQA Navigator prototype to reflect the
true threefold business goal:
```

1. Reduce routine ExamQA call volume from applicants/employees asking the same RDQA questions repeatedly.
2. Give applicants/employees a public, accessible, plain-English place to understand the RDQA process because RDQA is unique to Occu-Med and cannot be easily researched online.
3. Create an enhanced client-facing service that Occu-Med can charge employers for.

Add a new page called “Service Strategy” or “Executive Pitch.”

This page should explain:

- RDQA is valuable because it gives applicants the opportunity to provide additional medical information instead of being immediately disqualified.
- However, because RDQA is unique to Occu-Med, applicants cannot simply Google the process and understand what to do.
- This makes ExamQA the default help desk for routine questions.
- Applicants often call because they do not understand:
  - What RDQA means
  - Whether they are disqualified
  - What page 2 of the letter means
  - What provider to see
  - What their provider needs to write
  - Why “cleared to work” is not enough
  - What to do if they do not have insurance
  - What to do if they cannot afford testing
  - How to know if their documents are complete
  - How to request a deadline extension

Add a major feature category called “Enhanced RDQA Support.”

Enhanced RDQA Support should include:
- Plain-English RDQA process guide
- Condition-specific RDQA translator
- Provider packet generator
- Resource Navigator for low-cost care/testing options
- Document upload portal
- Documentation completeness review
- Missing-item feedback
- Deadline extension assistant
- Applicant case tracker

- Client operational dashboard

Add an applicant upload/completeness review workflow:
- Applicant uploads documents
- Applicant selects which condition each document addresses
- Portal checks against the condition checklist
- Analyst can mark missing items
- Portal generates feedback explaining what is missing
- Case receives a readiness score
- Case can be marked “Not ready,” “Needs applicant/provider correction,” “Ready for SME review,” or “Routed for SME review”

Add careful language:
- This is not a medical clearance tool.
- This does not provide medical advice.
- This does not determine qualification.
- This review is only to assess whether the submitted documentation appears to address the information requested in the RDQA letter before final review.

Add client revenue section:
Create three service tiers:

Basic RDQA Processing:
- Standard RDQA letter
- Applicant submits documents by email/fax
- Standard manual follow-up

Enhanced RDQA Support:
- Applicant portal
- Plain-English guidance
- Provider packets
- Resource Navigator
- Upload/completeness review
- Deadline extension support

Enterprise RDQA Management:
- Client dashboard
- Case movement analytics
- Delay reason reporting
- Branded client portal
- Monthly RDQA reports
- Priority workflow support

```
Add visual comparison:
Before RDQA Navigator:
- Applicant receives dense letter
- Applicant confused
- Calls ExamQA
- Provider submits vague letter
- Analyst sends clarification
- Deadline extension requested
- Client waits

After RDQA Navigator:
- Applicant opens portal
- Sees plain-English explanation
- Downloads provider packet
- Finds resource pathway
- Uploads documentation
- Receives completeness feedback
- Case moves to SME review faster
- Client sees operational status

Make this page polished, executive-demo ready, and visually
persuasive.
```

The Big Idea, Cleaned Up

What you are building is basically:

A TurboTax-style guided support system for RDQA.

TurboTax does not do your taxes magically. It walks you through what you need, explains confusing language, tells you what documents to gather, flags missing information, and gives you a path forward.

That is what RDQA needs.

Final Positioning

Not this:

"We made a help page."

This:

"We created a structured RDQA support platform that reduces manual analyst burden, helps applicants overcome access barriers, improves documentation quality, and gives clients a premium, trackable support service."

That is the pitch.